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School of Information and Communication Technology

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SOICT

PROJECT REPORT

MEDORA: Integrated Clinic Management and Electronic Medical Records System

Course: System Design

Class ID: 175882

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Abstract

MEDORA is a proposed integrated clinic-management and electronic medical-record (EMR) system for outpatient clinics and small healthcare facilities. It addresses workflow fragmentation across patient registration, appointment scheduling, check-in, clinical documentation, prescriptions, service execution, billing and operational reporting. The proposed solution centres on a shared patient and encounter record, explicit workflow states, role-based access control and an audit trail for sensitive actions. This report develops the business model, domain concepts, business rules, functional and non-functional requirements, use-case model and traceability needed to guide the subsequent system design and evaluation. The scope is deliberately limited to internal clinic operations; clinical decision-making, insurer settlement, national interoperability and production compliance certification remain outside the current project boundary.

Keywords: electronic medical records, clinic management, outpatient workflow, role-based access control, system analysis and design.

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1 Introduction and Problem Definition

1.1 Background and Existing System

Healthcare delivery is becoming a coordination problem as much as a clinical one. Across countries, providers are expected to offer safer, more continuous and more patient-centred care while dealing with rising demand, constrained staff capacity and increasingly complex information flows. The World Health Organization (WHO) estimates that approximately one in ten patients is harmed during health care and that more than half of this harm is preventable; medication-related harm accounts for a substantial share of the preventable cases [World Health Organization, 2023]. These figures do not imply that software can replace clinical judgement. They do, however, show why reliable identification, complete records, clear hand-offs and traceable actions are system-level concerns rather than administrative details.

Digital health is therefore moving from a collection of isolated applications towards an integrated operating capability. The WHO describes digital health as a means of aligning organizational, human and technological resources around better health outcomes, not simply as the deployment of another application [World Health Organization, 2021b]. Yet digitalisation can reproduce the same fragmentation when applications are implemented as disconnected, supply-driven tools; WHO identifies integration over isolation, data governance and protection of privacy as essential conditions for useful digital transformation [World Health Organization, 2021a; World Health Organization Alliance for Health Policy and Systems Research, 2024]. In a clinic, this distinction is important: an appointment calendar, a patient database and a billing screen may each work in isolation while the overall journey still depends on repeated data entry, informal communication and manual reconciliation. The result is a fragile process in which a patient can be correctly registered in one place but remain invisible to the next person who must act.

The persistence of legacy workflows is not unique to one country. The OECD reports that 75% of global fax traffic is used for medical services and that evidence-based innovations can take as long as 17 years to become routine practice, illustrating the distance between available technology and dependable day-to-day adoption [Organisation for Economic Co-operation and Development, 2023]. For a small clinic, the implication is practical: the priority is not to buy the most sophisticated technology, but to design a coherent system that staff can use consistently and that can exchange trustworthy data when the organization grows.

The Vietnamese context makes this challenge particularly tangible. Viet Nam entered the ageing phase in 2011; people aged 60 and over represented 11.9% of the population in 2019, and this share is projected to exceed 25% by 2050 [United Nations Population Fund Viet Nam, 2021]. An ageing population increases the need for repeated visits, longitudinal clinical histories and coordination across services. At the point of care, operational friction is already measurable. In a study of 7,931 outpatient visits at Thong Nhat Hospital, the mean total waiting time was 104.1 minutes, with departments handling roughly 150–250 patients per day while clinic capacity and staffing remained comparatively stable [Nguyen et al., 2018]. A separate study across Vietnamese health facilities reported average waits of 32.58 minutes at district hospitals

and 11.58 minutes at commune health centres before patients met clinical staff [Nguyen et al., 2020]. These are not merely comfort issues: long queues consume staff time, obscure workload, increase opportunities for transcription errors and weaken patients' confidence in the service.

1.2 Problem Statement and Motivation

The need for an electronic medical-record system is visible in three connected indicators. First, outpatient flow is slow: a study of 7,931 visits at a Vietnamese national hospital reported a mean total waiting time of 104.1 minutes, while departments handled approximately 150–250 patients per day without proportional capacity growth [Nguyen et al., 2018]. A broader study reported average waits of 32.58 minutes at district hospitals and 11.58 minutes at commune health centres before patients met clinical staff [Nguyen et al., 2020]. Second, patient-safety risk remains material: the WHO estimates that one in ten patients is harmed in health care and that more than half of this harm is preventable [World Health Organization, 2023]. Third, medication errors alone are estimated to cost about US$42 billion globally each year [World Health Organization, 2017].

These figures do not prove that every clinic has the same performance. They establish why the workflow deserves system-level attention. When identity, appointments, clinical notes, orders and payment are split across paper forms, spreadsheets or isolated applications, the following problems become likely:

- duplicate patient records and repeated demographic entry;
- appointment, room or staff conflicts that are discovered too late;
- incomplete visibility of allergies, previous encounters, current medication or open orders;
- unclear responsibility when an order moves from prescription to execution;
- silent overwriting of signed information and weak evidence of who changed it; and
- limited management visibility into waiting, workload, service progress and payment status.

WHO identifies fragmented data governance and weak exchange mechanisms as barriers to timely, high-quality use of health data [World Health Organization, 2021a]. MEDORA addresses this coordination gap by providing one patient identity, one encounter context and explicit workflow states across authorized roles. Its "smart" behaviour is deliberately bounded: the system applies rules, warnings, validation and audit trails at the right step, while diagnosis and treatment decisions remain with qualified clinicians.

The architecture is part of the motivation. Separating presentation, application rules, domain entities and persistence prevents a new screen from bypassing clinical or access-control rules. Explicit services and state transitions make double-booking prevention, signed-record amendment, authorization and auditability testable. The result is a system that is safer to operate today and easier to extend tomorrow, rather than a set of screens that merely digitizes existing fragmentation.

1.3 Project Objectives and Success Criteria

This project proposes MEDORA, an integrated clinic-management and electronic medical-record system for a clinic or small healthcare facility. Its objective is to turn a fragmented sequence of tasks into a controlled, observable and reusable workflow. The project pursues five linked outcomes:

- one stable patient identity and one longitudinal record across visits;
- coordinated appointments, queues, rooms and staff;
- structured support for examination, diagnosis, prescriptions and service orders;
- auditable handling of sensitive changes and role-based access to health data; and
- operational dashboards and reports that support evidence-based management.

Success is specified using SMART criteria so that the prototype can be evaluated without overstating clinical impact. During the final test cycle, against a documented baseline workload, MEDORA will:

- reduce median registration-to-completion time by at least 25% for the representative outpatient scenario;
- accept zero double-bookings for the same doctor, room or constrained resource across 100% of the concurrency test cases;
- achieve at least 95% completion of configured mandatory encounter fields across the tested completed visits;
- record an actor, timestamp, action and affected entity for 100% of tested sensitive events, including clinical amendments, prescription changes, exports and permission changes; and
- demonstrate that 100% of negative authorization test cases are rejected at the API/server boundary, while the principal happy-path workflow completes successfully in the agreed test environment.

Each target is specific to a defined workflow, measurable through test evidence, achievable for the course-project scope, relevant to the stated pain points and time-bound to the final evaluation cycle. The figures are prototype acceptance targets, not claims of production performance; a real deployment would first establish a site-specific baseline and then revalidate them with clinical and operational stakeholders.

1.4 System Scope, Assumptions, and Constraints

MEDORA is an internal web system for a clinic or small healthcare facility. It supports the outpatient journey from patient identification and appointment or walk-in reception to check-in, screening, consultation, prescription or service order, execution, encounter closure and payment status.

Receptionists manage identity, appointments and queues; nurses record screening; doctors own clinical decisions; technicians and medicine staff record actual execution; cashiers record financial events; managers, administrators and auditors supervise operations, configuration and accountability. The system preserves the difference between planned and completed work: an appointment is not an encounter, a prescription is not a medicine event, an order is not a result, and an invoice is not a payment.

The MVP focuses on internal operations, basic billing, reporting and governance. Patient self-service, external diagnostic systems, insurer settlement and advanced inventory are later-stage extensions. Every sensitive action is constrained by role, scope and workflow state; signed notes are amended rather than overwritten, and concurrent booking must not create a conflict.

1.5 Proposed Solution and System Overview

MEDORA is a clinic operating system built around one shared patient and encounter record, not a collection of unrelated modules. Its central unit of work is the outpatient encounter. A Patient may have many Appointments and Encounters; an Encounter is the controlled context for clinical notes, vital signs, diagnoses, prescriptions and procedure orders; an order may consume a scheduled ResourceBooking and later receive an execution result; and an Invoice and Payment record the financial consequence without changing the clinical history. Attachments, notifications and AuditLogs are linked to the relevant business object rather than stored as anonymous files or messages.

Table 1 defines the product by capability. Each row states the function, its primary users and the operational value it is expected to create. This makes the product boundary concrete before the report describes its technical realization: MEDORA is an internal clinic-management and electronic medical-record product that coordinates people, information, resources and decisions throughout an outpatient episode.

Table 1: MEDORA product definition and value proposition

Function	Primary users	What the system does	Value delivered
Patient identity and record management	Reception, nurses, doctors	Search/register patients, flag duplicates and link history, allergies, documents and encounters.	One longitudinal record and safer identification.
Appointment, schedule and queue management	Reception, managers; patient portal (Phase 2)	Book, reschedule, cancel, check in and assign queues against current availability.	Visible workload and fewer coordination conflicts.

Function	Primary users	What the system does	Value delivered
Clinical examination and EMR	Nurses, doctors	Capture screening, vital signs, consultation, diagnosis and follow-up in the Encounter.	Continuous, structured clinical context.
Prescription and medication safety	Doctors, medicine staff	Issue prescriptions with directions and configured safety checks; retain amendment and medicine-event history.	Safer hand-offs between prescribing and dispensing.
Procedure, service and resource coordination	Doctors, nurses, technicians	Create orders, allocate constrained resources and record execution/result.	Prevents double-booking and links decision to outcome.
Billing and payment status	Cashiers, managers	Create invoices and retain payment/refund history separately from clinical content.	Clear financial accountability and reconciliation.
Administration and role-based access	Administrators, managers, auditors	Manage users, permissions, catalogues, schedules and scope.	Least privilege and adaptable operations.
Dashboards, reports and audit trail	Managers, auditors	Present operational indicators, controlled export and sensitive-event history.	Evidence for decisions and investigation.
Attachments and notifications	Assigned staff; patient portal (Phase 2)	Store controlled documents and send privacy-aware operational notifications.	Reliable follow-up with lower privacy exposure.

The solution uses four cooperating layers:

- **Presentation:** role-specific workspaces for reception, nursing, consultation, service execution, cashier operations and management.
- **Application:** use cases such as registration, booking, check-in, consultation, prescription, service scheduling, result recording, encounter closure and payment.
- **Domain:** rules for availability, permissions, state transitions, signed-record amendments and clinical warnings.
- **Data:** persistent patient, encounter, order, resource, invoice and audit entities with integrity constraints and history.

Each workflow step has an explicit state and accountable actor. Backend authorisation, controlled attachments and audit history protect the record while the modular structure leaves a clear path for later portal, diagnostic and interoperability extensions.

2 Business Analysis and Domain Modeling

2.1 Business Context and Operational Overview

MEDORA is an internal web system for an outpatient clinic or small healthcare facility. It coordinates one outpatient episode from patient identification and appointment handling to consultation, service execution, payment status and operational reporting. Its central design principle is continuity: every authorised role works from the same patient and encounter context rather than from disconnected records.

Four distinctions protect that continuity. An Appointment reserves time, while an Encounter represents the visit that actually occurs. A prescription is a clinical decision; dispensing or administration is a later, accountable event. A service order is not an execution result. An Invoice records charges, whereas a Payment or refund records a financial transaction. These distinctions prevent the system from reporting planned work as completed work.

The MVP covers internal clinic operations, basic billing, reporting and governance. Patient self-service, external laboratory or imaging interfaces, insurer settlement and other integrations are later-stage capabilities.

Business area	Target operating model
Patient identity	Search before registration; flag possible duplicates; preserve merge history.
Visit coordination	Treat appointment, check-in, encounter and queue placement as explicit states.
Clinical care	Keep screening, documentation, diagnosis, prescription and orders in one encounter context.
Execution and finance	Record allocation, service execution, invoice and payment as separate events.
Governance	Apply scoped access, controlled export and auditable sensitive actions.

Table 2: Business baseline and target operating model

2.2 Stakeholders and Responsibilities

Table 3 defines the roles that participate in the target workflow. Access is always restricted by role, facility, assignment and workflow state; managerial responsibility does not create unrestricted clinical-edit access.

Role	Primary responsibility	Authority boundary
Receptionist	Identify/register patients; manage appointments, arrivals and queues.	Administrative and operational data only.
Nurse	Record screening, vital signs and assigned care activity.	Cannot alter diagnosis or signed prescriptions.
Doctor	Consult, document, diagnose, prescribe, order services and close encounters.	Owns clinical decisions for assigned encounters.
Technician / Pharmacist	Execute assigned service work or record the medicine event.	Records execution; does not create the clinical decision.
Cashier	Create eligible invoices and record payments or refunds.	Cannot edit clinical content.
Manager	Monitor capacity, services, revenue and reports.	Uses scoped operational information.
Administrator / Auditor	Configure access/catalogues or review audit evidence.	Administration and audit are separate from clinical authority.
Patient (Phase 2)	Request appointments and view released documents.	Accesses only verified, explicitly released information.

Table 3: Stakeholder responsibilities and authority boundaries

Actor	Main system interaction	Boundary
Receptionist	Patient search, registration, appointment, check-in and queue.	No clinical decision editing.
Nurse / Doctor	Assigned encounter, clinical history, screening, documentation and orders.	Access is assignment- and role-scoped.
Technician / Pharmacist	Assigned service execution or medicine event.	Cannot create the originating clinical decision.
Cashier	Invoice, payment, refund and payment status.	Cannot modify clinical records.
Manager / Administrator / Auditor	Reports, configuration or audit review.	Each role has a separate scope.
Patient (Phase 2)	Appointment request and released-document viewing.	Verified self-service only.

Table 4: Role-to-activity mapping and interaction boundaries

2.3 Business Workflow and Controls

Figure 1 shows the normal outpatient journey. Figure 2 isolates appointment and arrival exceptions, where invalid transitions are most likely to disrupt service flow.
The workflow is governed by five control points:

1. Search and duplicate checks occur before a new patient record is created.
2. Booking and rescheduling validate provider, room and required-resource availability before commitment.
3. Check-in creates one valid active Encounter and its queue state.
4. Orders, execution and medicine events are recorded as separate lifecycle events.
5. Encounter closure, payment and refund preserve their own accountable history.

Process	Completion state	Alternative or exception
Identity and booking	Confirmed patient and valid appointment.	Duplicate review or an alternative slot; no conflict is committed.
Arrival and queue	One active encounter with queue status.	Accepted walk-in or rejected duplicate active encounter.
Consultation and documentation	Structured clinical note and plan.	Draft is retained; signed content is amended, not overwritten.
Order and execution	Order, allocation and recorded result.	Unavailable resource, delay or cancellation preserves status history.
Closure and finance	Encounter summary plus invoice/payment status.	Incomplete work blocks closure; refund retains reason and history.

Table 5: Major business processes, outcomes and controlled alternatives

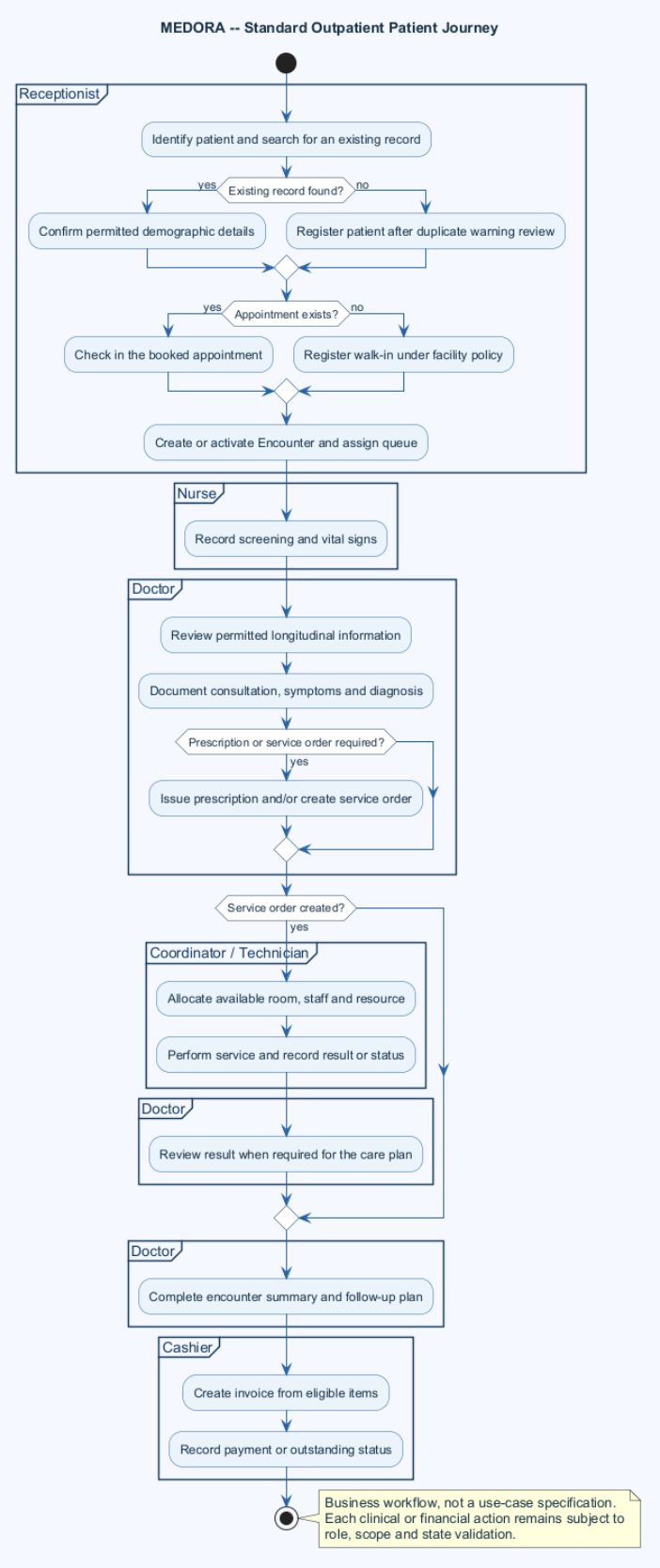


Figure 1: Target business workflow for a standard outpatient journey

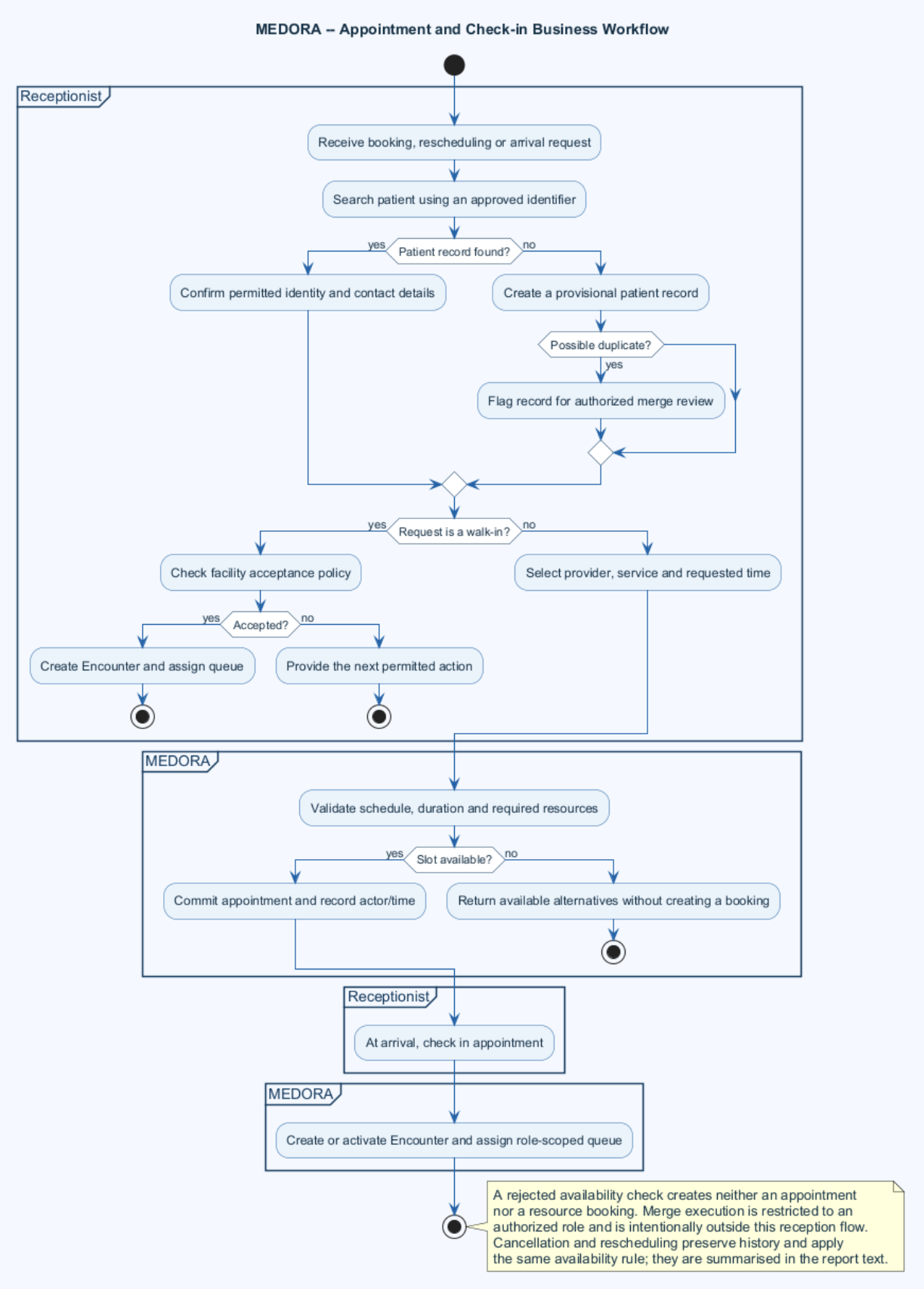


Figure 2: Appointment and check-in workflow with alternative and exception paths

2.4 Business Rules

ID	Rule	Required behaviour
BR-001	Patient identity	Use a stable identifier, flag possible duplicates and preserve merge history.
BR-002	Availability	Validate provider, room, staff and required resources before booking or allocation.
BR-003	Encounter integrity	Prevent duplicate active encounters and maintain role-scoped queues.
BR-004	Authorisation and audit	Enforce role, scope and state checks at the server boundary; retain sensitive-event evidence.
BR-005	Clinical-record integrity	Preserve signed notes; record corrections as linked amendments with a reason.
BR-006	Clinical safety and execution	Warnings inform clinicians without deciding treatment; prescriptions, orders and actual execution remain distinct.
BR-007	Financial history	Keep invoices, payments and refunds separate from clinical records and retain transaction history.

Table 6: Core business rules

2.5 Domain Model and Core Entities

Figure 3 presents the clinical core. The remaining operational and governance concepts extend this core without changing its central relationship: the Encounter contextualises the clinical work performed for one Patient.

Entity group	Purpose
Patient and MedicalRecord	Stable identity and longitudinal clinical context.
Appointment and Encounter	Planned visit versus actual outpatient episode.
ClinicalNote, Diagnosis and Prescription	Authorised clinical documentation and decisions.
ProcedureOrder and ResourceBooking	Requested service, constrained allocation and execution context.
Invoice and Payment	Charges and financial transactions without clinical mutation.
UserAccount, Role, Attachment and AuditLog	Scoped access, controlled documents and accountability evidence.

Table 7: Core entity glossary

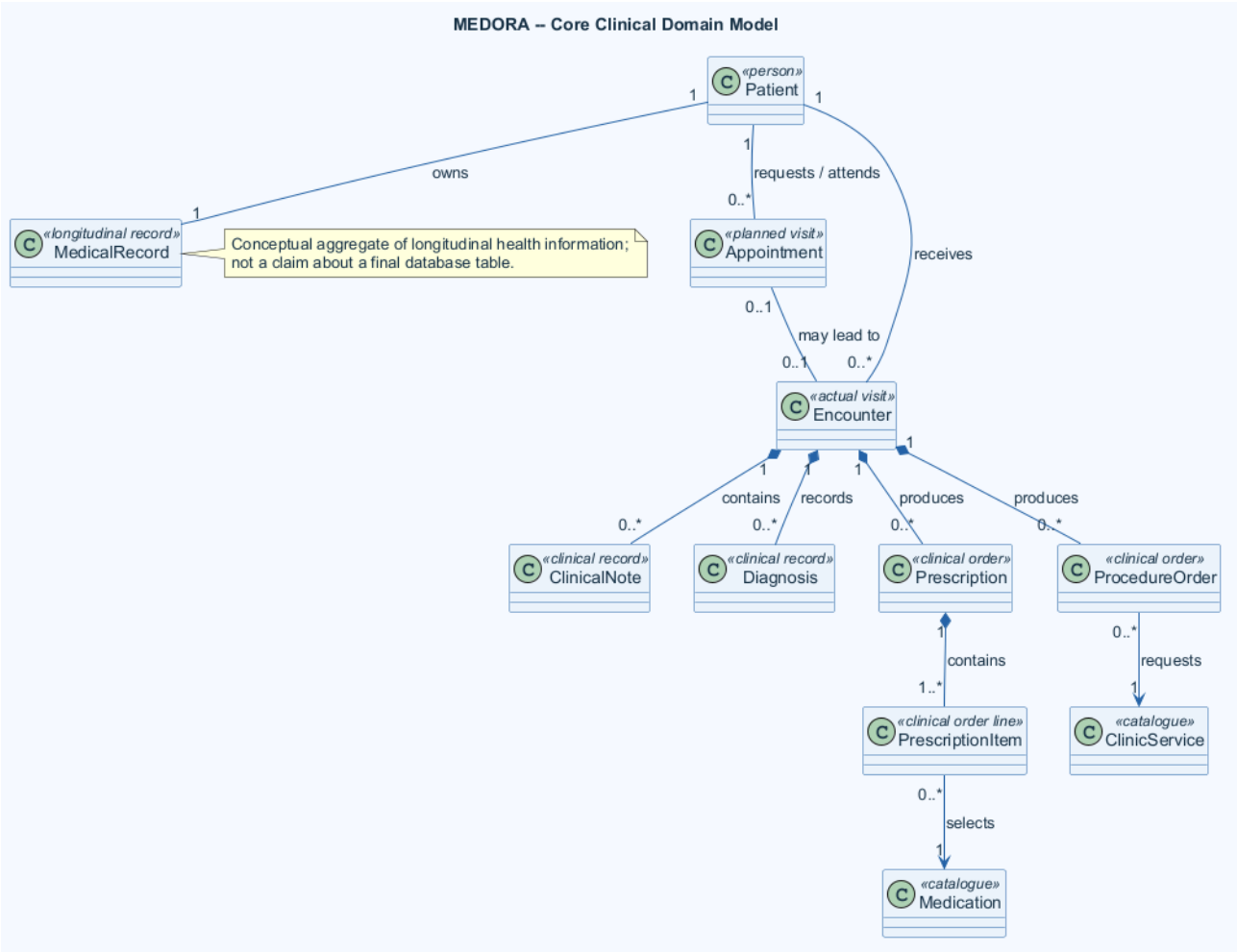


Figure 3: Core clinical conceptual domain model

Relationship	Business meaning
Patient–Appointment–Encounter	A patient may have many appointments and encounters; an appointment may lead to one encounter, while a walk-in has none.
Encounter–clinical records	Notes, diagnoses, prescriptions and procedure orders belong to the encounter in which they are created.
Prescription–medicine event	A prescription contains medicine directions; dispensing or administration is a separate, accountable event.
ProcedureOrder–ResourceBooking–Result	An order may reserve resources and later obtain an execution result; neither is implied by the other.
Invoice–Payment	One invoice may have zero or many payment, refund or adjustment transactions.
UserAccount–Role–AuditLog	Role and scope govern access; audit evidence records the actor, action, target and time.

Table 8: Core domain relationships

Business need	System implication	Trace
Reliable identity	Search, duplicate control, merge history and longitudinal record.	BR-001; FR-003–FR-004
Conflict-free coordination	Transactional availability, queue state and resource allocation.	BR-002–BR-003; FR-005–FR-006, FR-015
Accountable care	Signed-note amendments, warnings and separate execution events.	BR-005–BR-008; FR-009–FR-018
Financial accountability	Invoice, payment and refund history separate from clinical data.	BR-007; FR-022–FR-023
Governance	Least privilege, audit trail, reports and controlled export.	BR-004; FR-026–FR-030

Table 9: Business-to-system traceability

3 System Requirements Analysis

3.1 Functional Decomposition and Package Structure

Figure 4 groups MEDORA by business capability rather than user interface. It provides the functional boundary for the requirements and use-case model.

Identity and Access provides the stable patient and user context on which all other capabilities rely. Scheduling and Reception controls planned appointments, walk-ins, queues and resource availability. Clinical Care records the actual encounter; Orders and Execution preserves the distinction between a decision and the action carried out. Finance and Operations handles eligible charges and operational visibility, while Governance applies cross-cutting controls for attachments, audit evidence and exports. The package dependencies are deliberately not shown in Figure 4: they would duplicate the logical architecture and reduce readability.

3.2 Actors and Overall Use Case Model

The overall use-case model in Figure 5 gives a bounded view of actor goals. Patient self-service is explicitly Phase 2; in the MVP, reception staff perform appointment and check-in work on the patient's behalf. No insurer, payment gateway, laboratory information system, imaging system or messaging provider is shown as an actor because no approved integration contract exists.

3.3 Use Case Diagrams by Functional Group

The focused diagrams below decompose the overview without changing the actor boundary. Solid associations identify participating external roles. A dashed `«include»` relationship is mandatory reusable behaviour; `«extend»` is conditional behaviour that leaves the base use case valid. UC-054 (operational notification) and UC-055 (released-document viewing) are explicitly scoped as P1/Phase 2 capabilities and are not implementation claims. The sources and mappings are maintained in the model inventory under `doc/asset/03-system-requirements/use-case-model`.

3.3.1 Patient Management

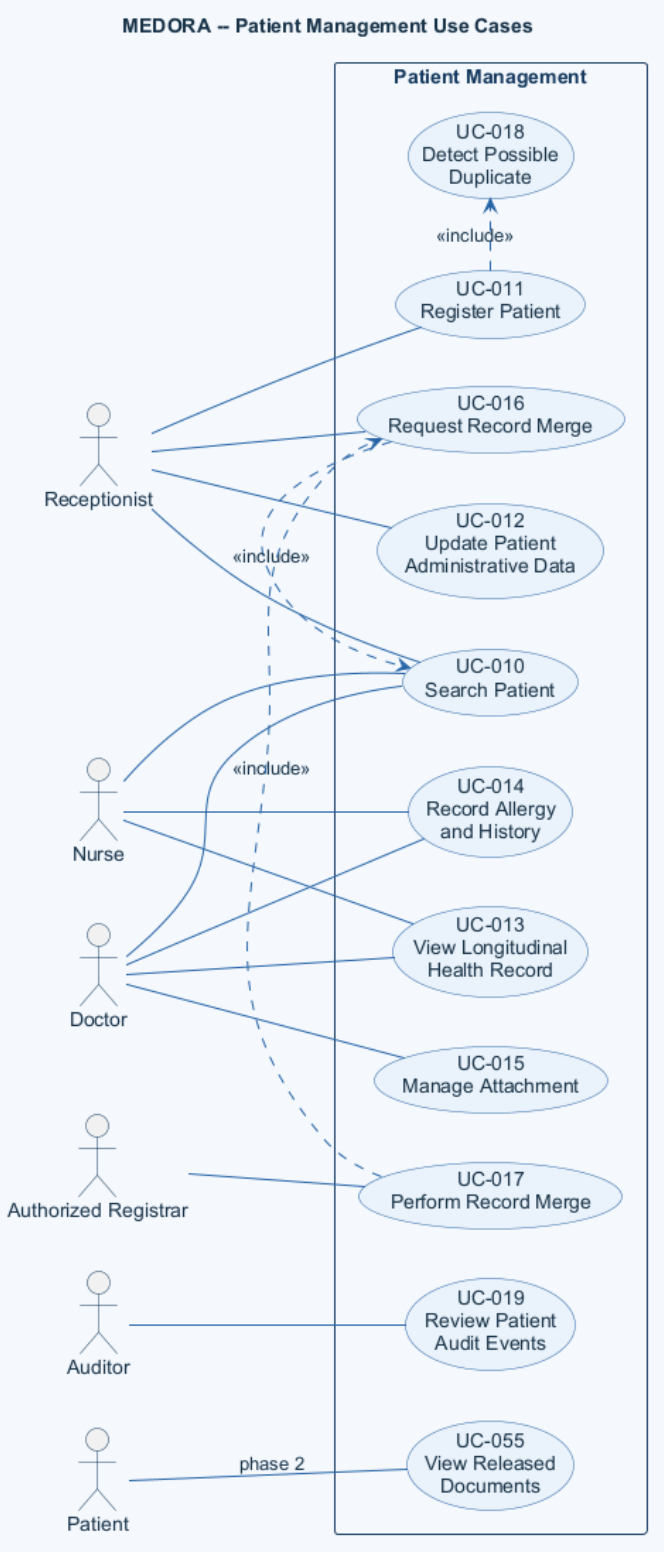


Figure 6: Patient-management use cases

3.3.2 Appointment, Check-in and Queue Management

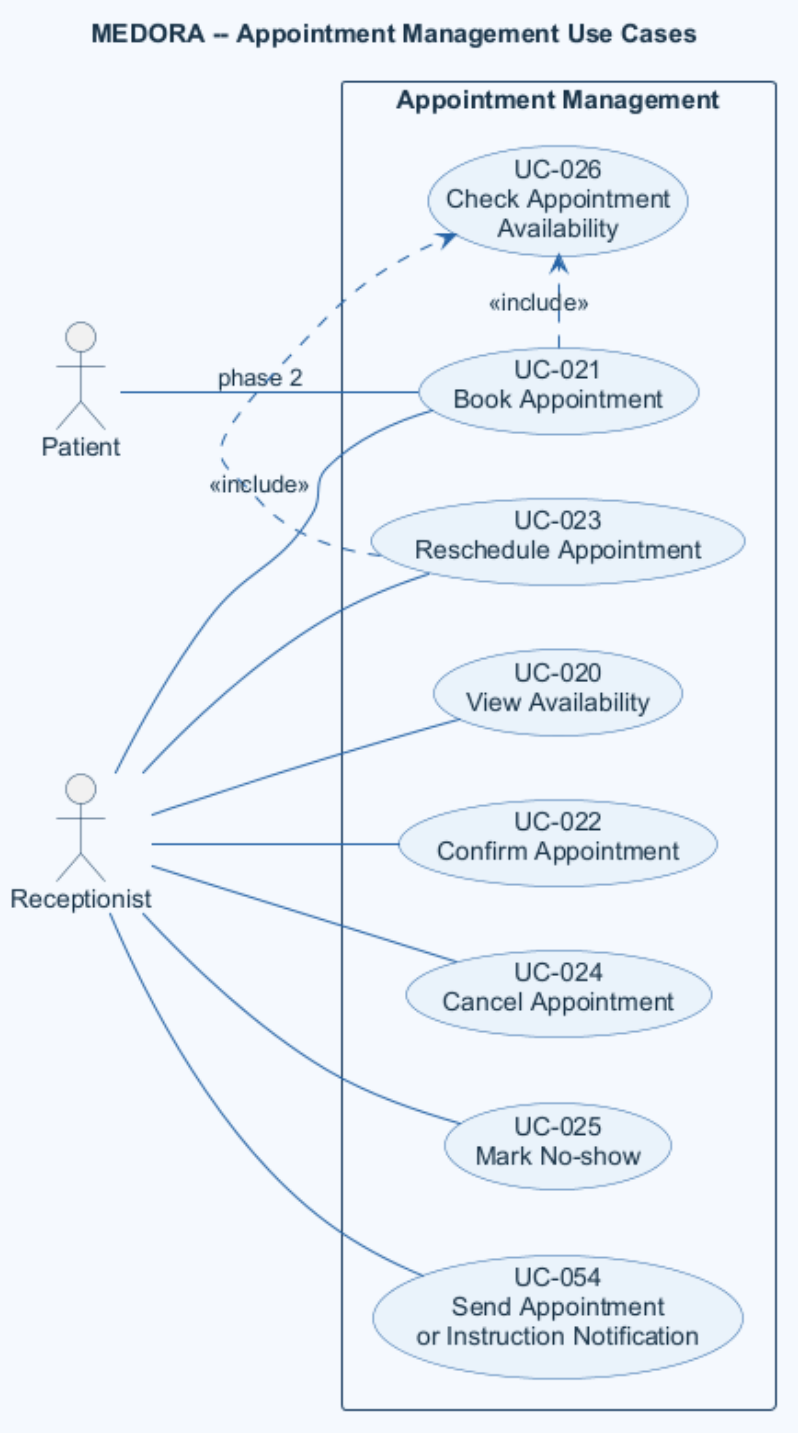


Figure 7: Appointment-management use cases

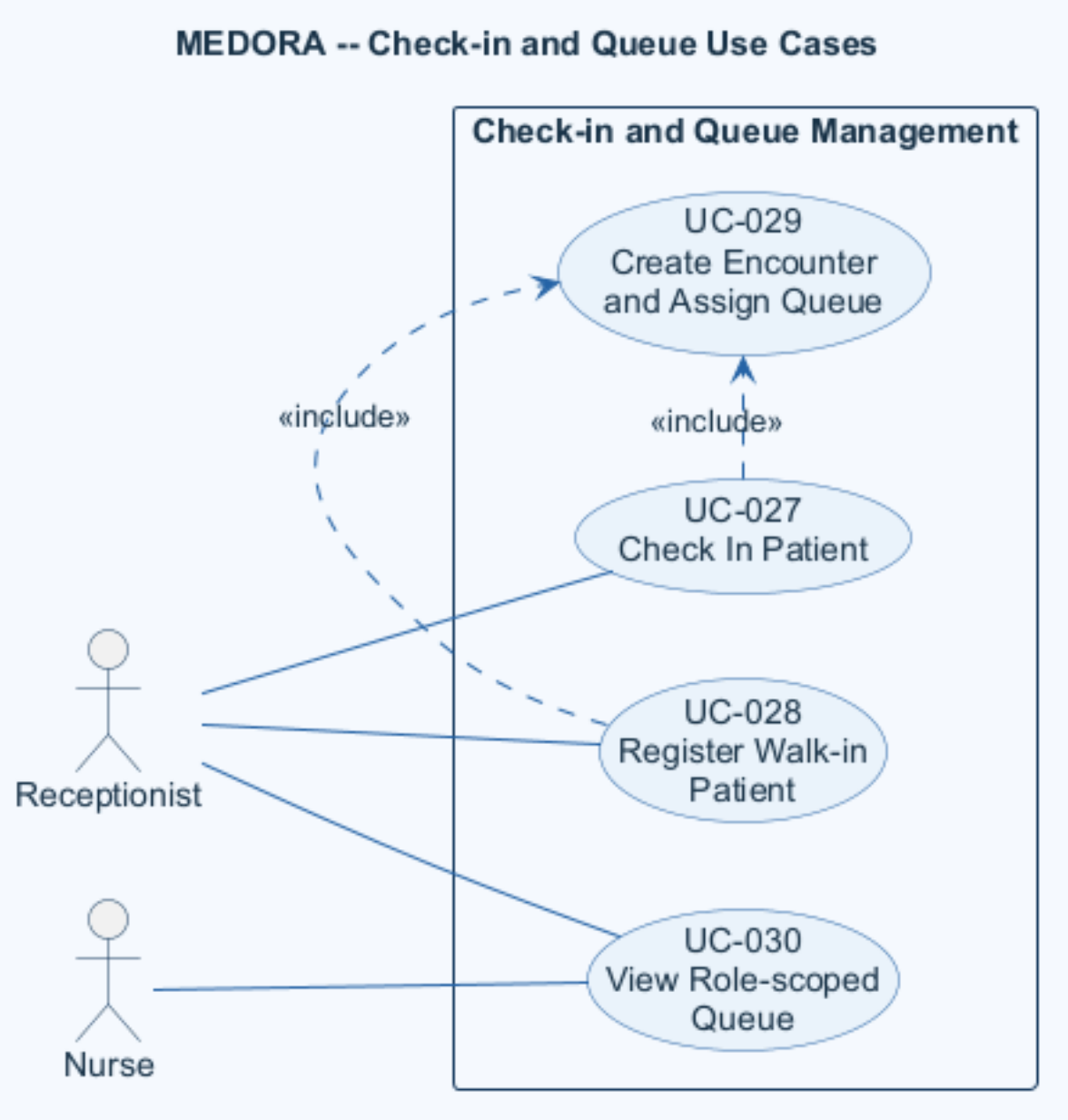


Figure 8: Check-in and queue-management use cases

3.3.3 Medical Examination and Encounter Management

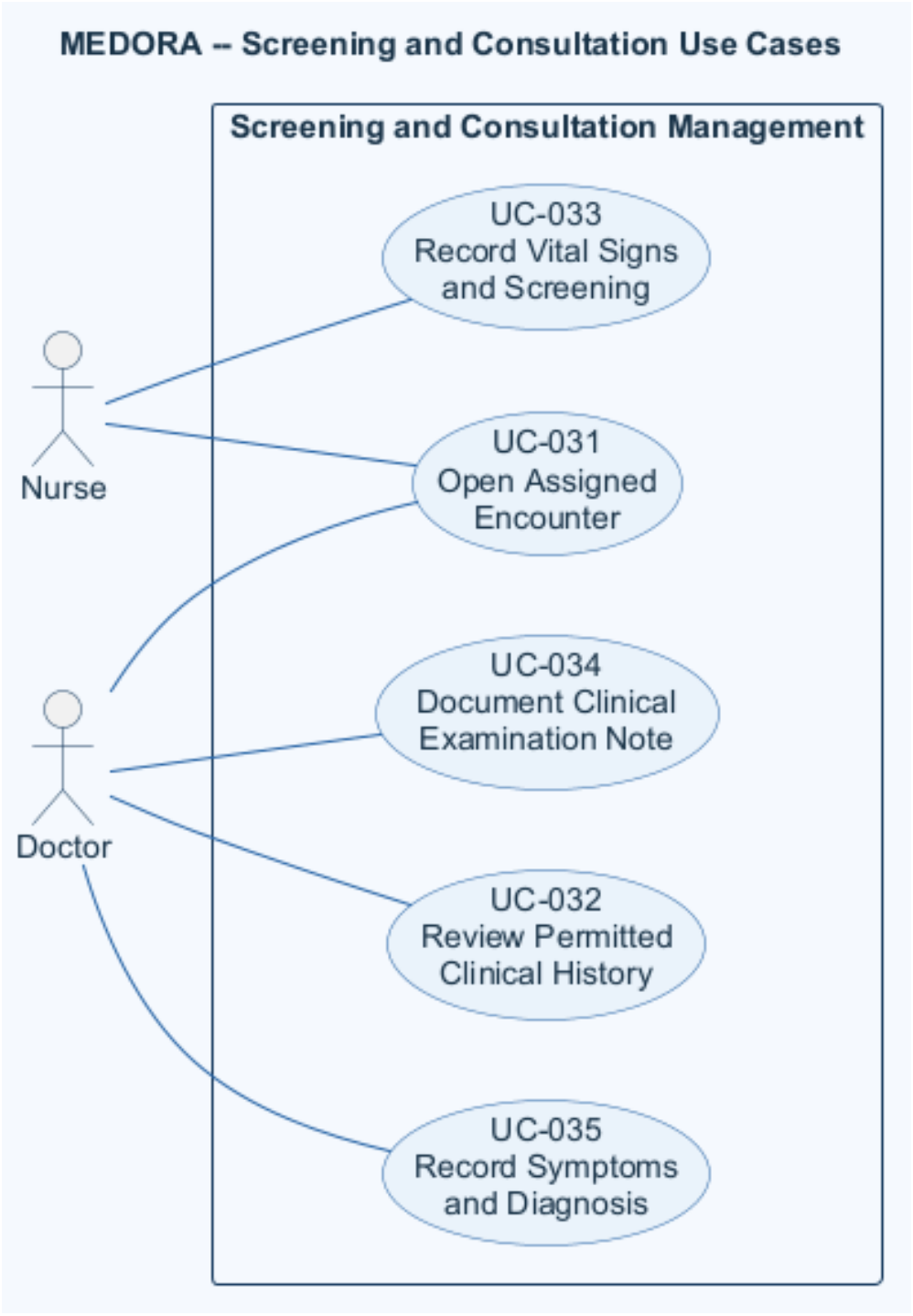


Figure 9: Screening and consultation use cases

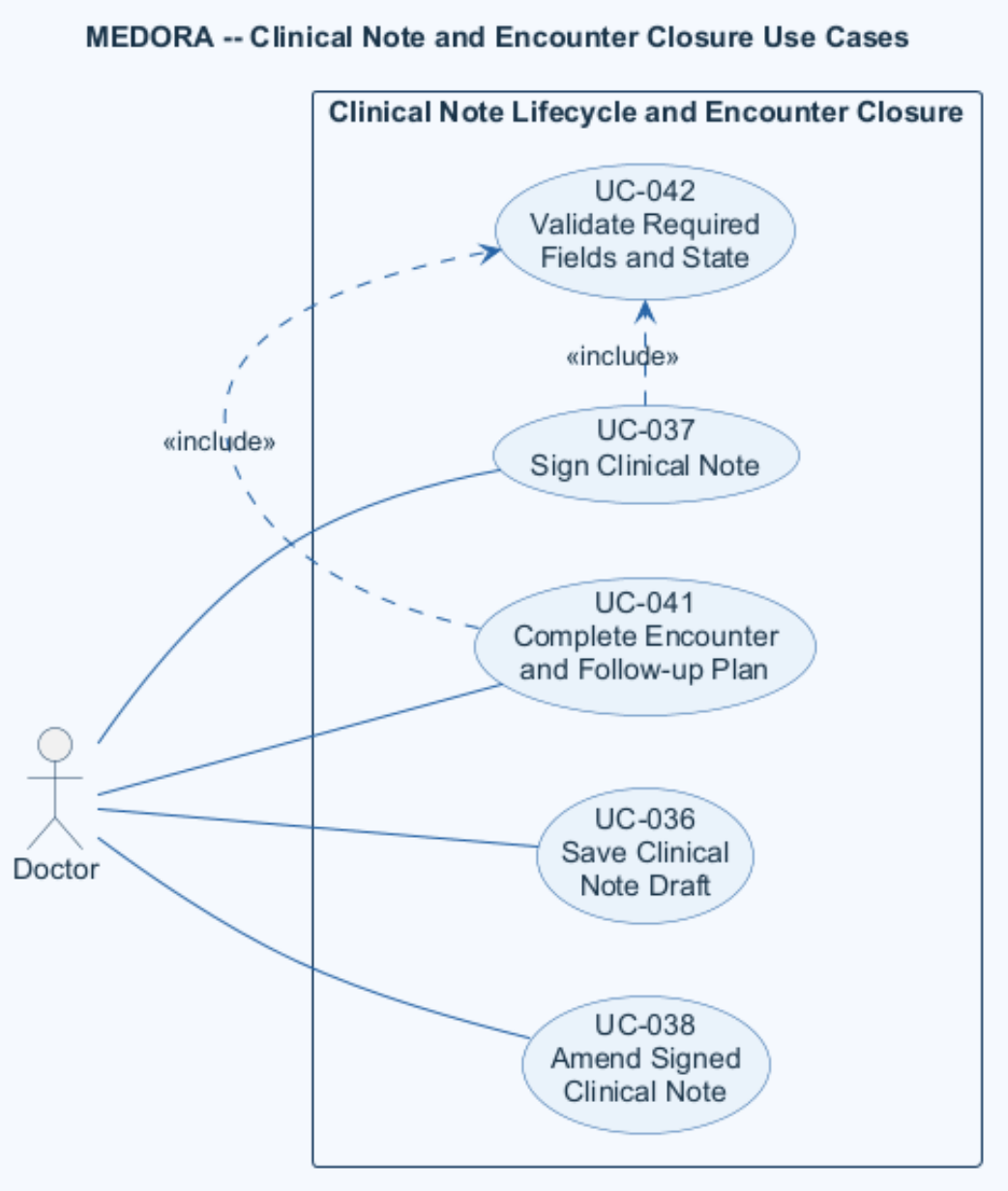


Figure 10: Clinical-note lifecycle and encounter-closure use cases

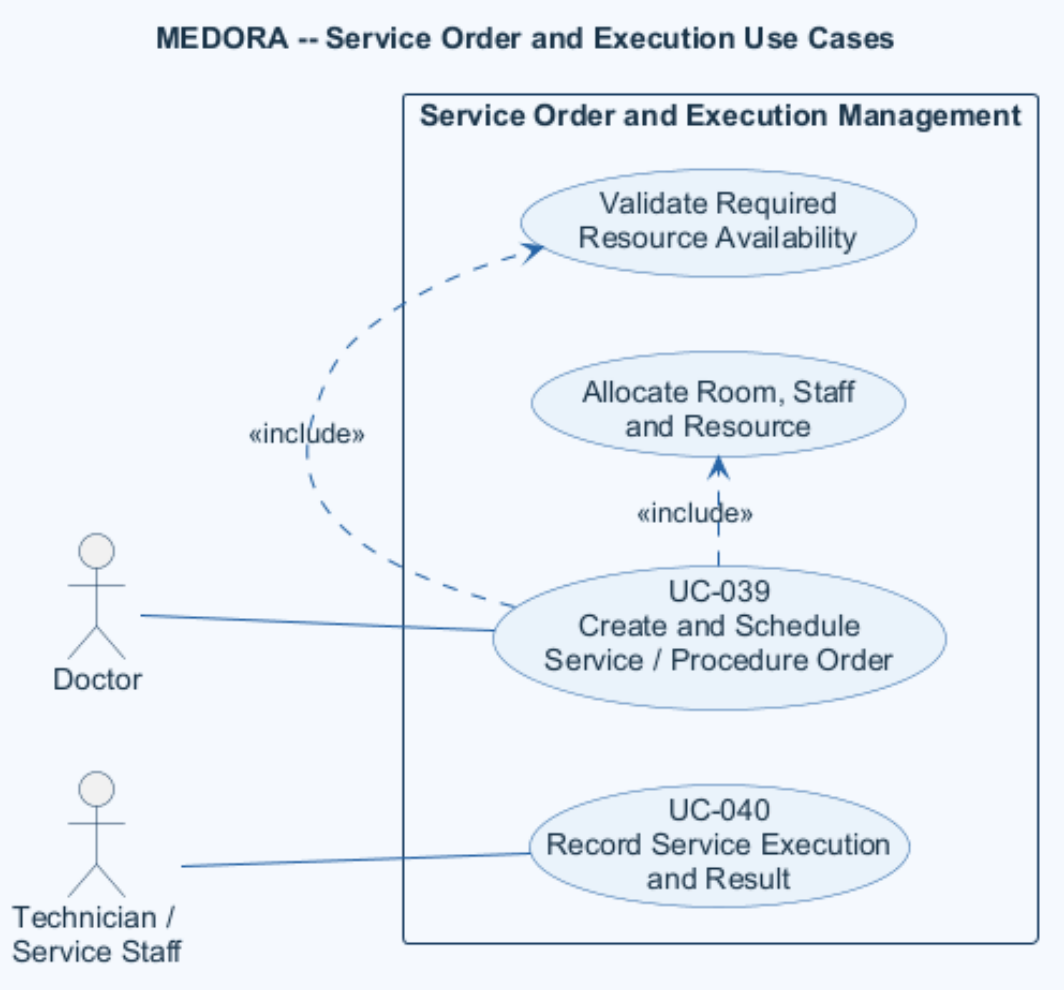


Figure 11: Service-order and execution use cases

3.3.4 Prescription, Billing and Governance

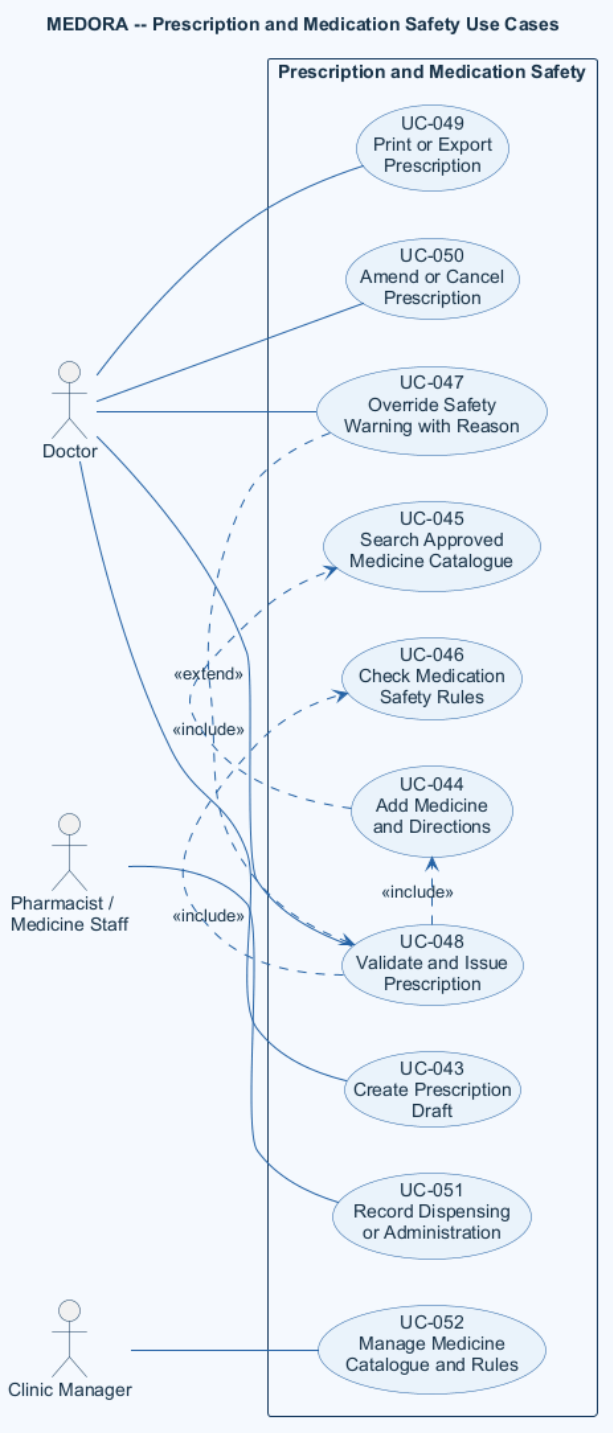


Figure 12: Prescription and medication-safety use cases

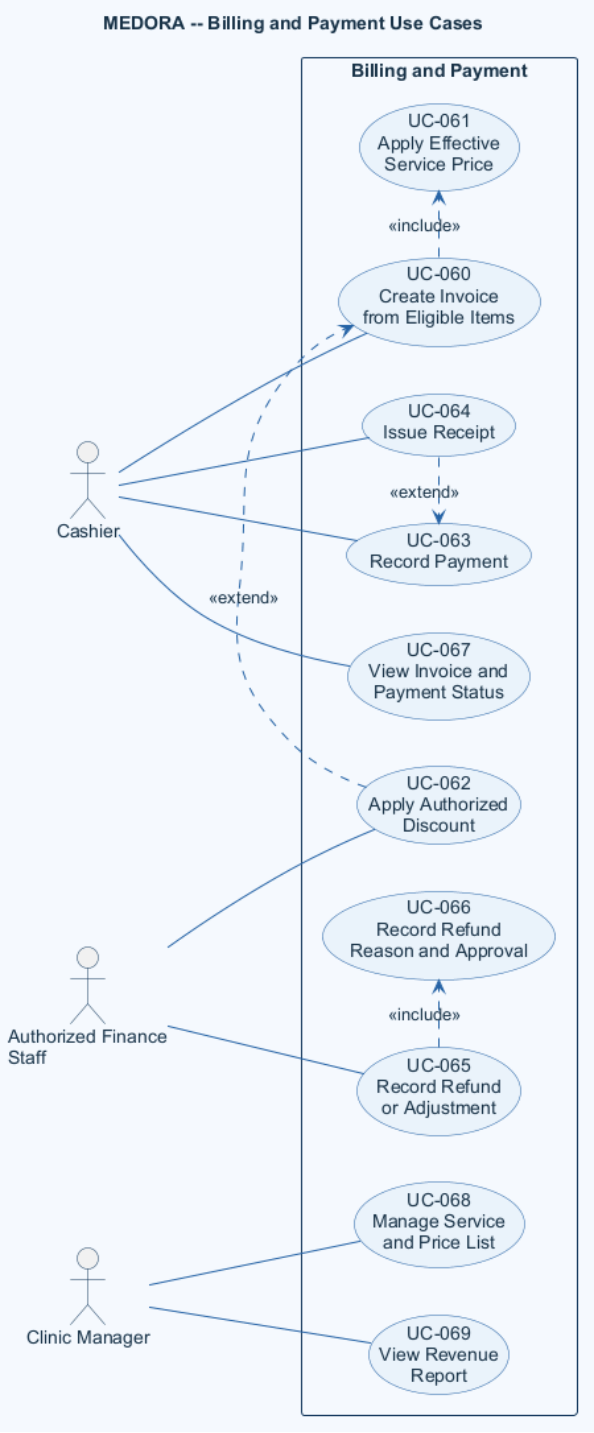


Figure 13: Billing and payment use cases

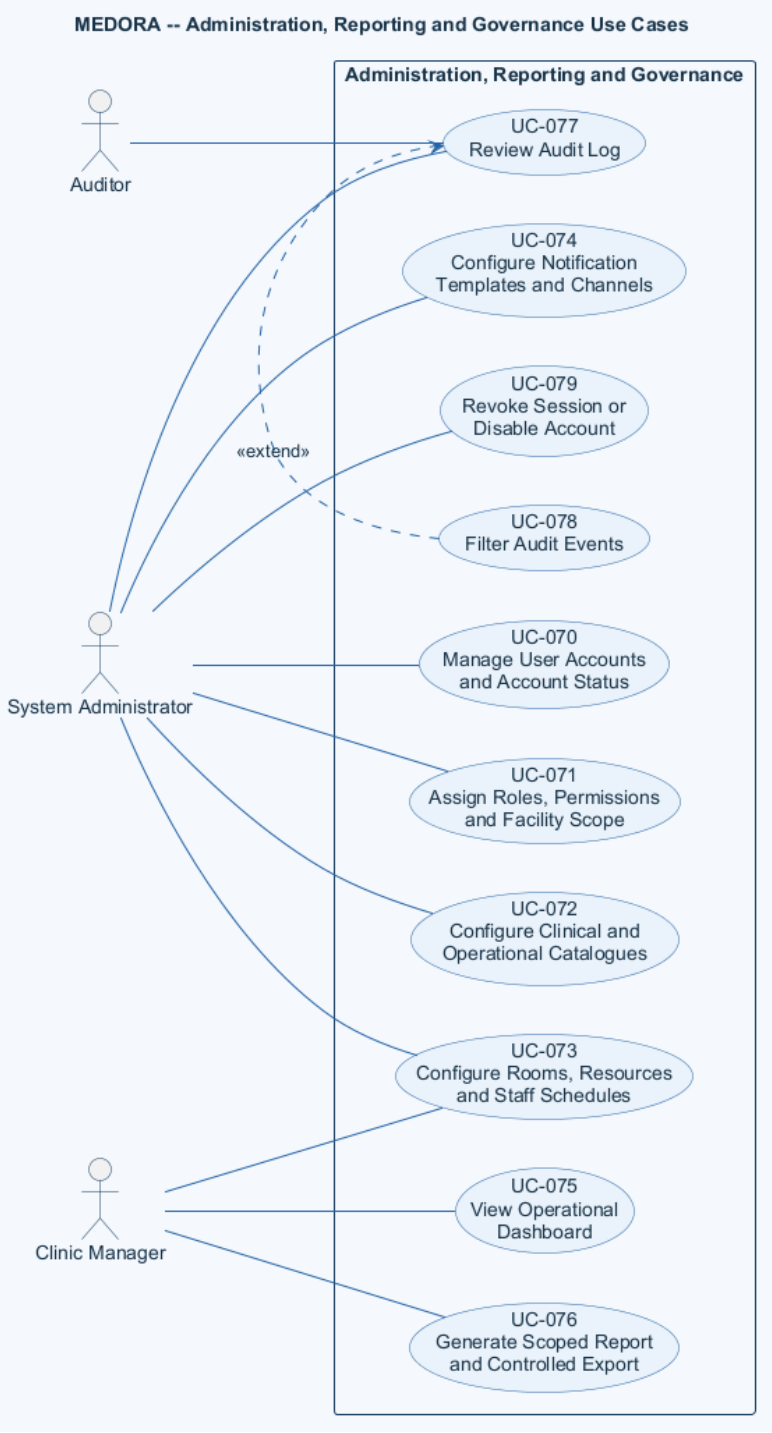


Figure 14: Administration, reporting and governance use cases

3.4 Detailed Use Case Specifications

The six use cases below carry the principal state, safety or financial risk. Each specification states the minimum successful path and the control that prevents an invalid state transition.

3.4.1 UC-021 Book Appointment

Item	Specification

Actor, trigger and preconditions	Receptionist; a patient and service/provider context are selected; the actor is authorised.
Success flow	Select slot; validate provider, room, duration and required resources; atomically create the Appointment and audit-relevant metadata.
Alternative and exception flow	Rescheduling repeats validation and preserves history. An unavailable slot, unresolved identity or invalid scope creates no appointment.
Postconditions and data	One valid Appointment exists; no Encounter is created by booking alone.
Rules and trace	BR-002, BR-004; FR-005, FR-017; UC-026; NFR-06, NFR-15.

3.4.2 UC-027 Check In Patient

Item	Specification
Actor, trigger and preconditions	Receptionist receives an identified booked patient or an accepted walk-in.
Success flow	Confirm arrival; check the active-encounter rule; create/activate the Encounter; assign the role-scoped queue state.
Alternative and exception flow	An accepted walk-in follows the same queue rule. A conflicting active encounter, rejected walk-in or invalid scope creates no duplicate encounter.
Postconditions and data	The relevant Appointment is updated and one active Encounter with queue status is available to authorised roles.
Rules and trace	BR-003, BR-004; FR-006; UC-029; NFR-06, NFR-15.

3.4.3 UC-037/UC-038 Sign and Amend a Clinical Note

Item	Specification
Actor, trigger and preconditions	Authorised Doctor; a draft note belongs to an assigned Encounter.
Success flow	Validate required content and state; sign the note with actor/time. A correction creates a linked amendment with a reason.
Alternative and exception flow	A draft may be saved for later. Missing fields, invalid state, invalid scope or a concurrent update prevents signing or requires reconciliation.
Postconditions and data	Signed content remains intact; each amendment retains author, time, reason and linkage.
Rules and trace	BR-004, BR-005; FR-009, FR-020, FR-032; UC-042; NFR-06, NFR-07, NFR-15.

3.4.4 UC-048 Validate and Issue Prescription

Item	Specification
Actor, trigger and preconditions	Authorised Doctor requests issue for a valid encounter with complete medicine items.
Success flow	Validate dose and directions; evaluate configured safety rules; address a warning or record a permitted override; issue the prescription.
Alternative and exception flow	The Doctor may retain a draft. Missing fields, an unresolved required warning or invalid scope blocks issue; unavailable source data does not create an invented warning.
Postconditions and data	An issued prescription and item history exist; no dispensing or administration event is inferred.
Rules and trace	BR-006, BR-008; FR-011–FR-013; UC-046–UC-047; NFR-06, NFR-07.

3.4.5 UC-039/UC-040 Service Order, Allocation and Result

Item	Specification
Actor, trigger and preconditions	Doctor creates an order for an active encounter; assigned Technician/Service Staff records execution.
Success flow	Record service, reason and priority; validate/allocate required room, staff and resource; record performer, time, status and result.
Alternative and exception flow	An order may be scheduled later or remain pending. Unavailable resources, unassigned staff, delay or cancellation preserve a valid non-completed state.
Postconditions and data	ProcedureOrder, optional ResourceBooking and execution/result history are linked to the Encounter.
Rules and trace	BR-002, BR-004, BR-007; FR-014–FR-018; NFR-06, NFR-15.

3.4.6 UC-063/UC-065 Record Payment and Refund

Item	Specification
Actor, trigger and preconditions	Cashier records payment for an eligible Invoice; authorised Finance Staff records a refund or adjustment.
Success flow	Verify outstanding status; record method, amount and time; update the Invoice. Refunds record the prior transaction, reason and required approval.

Alternative and exception flow	Partial payment leaves an outstanding balance. Invalid amount/status, invalid scope or duplicate submission produces no financial effect.
Postconditions and data	Payment/refund history and invoice status are updated; clinical content is unchanged.
Rules and trace	BR-004, BR-007; FR-022–FR-023; UC-064, UC-066; NFR-06, NFR-07, NFR-15.

3.5 Functional Requirements

Table 16 groups related requirements without changing their identifiers. P0 is required for the MVP; P1 is a controlled extension; P2 depends on a later organisational decision or integration contract.

Table 16: Functional requirements catalogue

ID	Capability	Observable system behaviour	Priority
FR-001–FR-002	Authentication and access	Authenticate individual accounts; manage roles, facility scope and sensitive-session controls; enforce authorisation at the server boundary.	P0
FR-003–FR-004	Patient identity	Create/search patient profiles, flag duplicates and preserve authorised merge evidence.	P0/P1
FR-005–FR-006	Appointment and reception	Manage appointment status, availability, walk-ins, check-in and role-scoped queues.	P0
FR-007–FR-010	Encounter documentation	Record screening, history, clinical notes, symptoms, diagnoses and follow-up in encounter context.	P0
FR-011–FR-013	Prescription and medicine event	Validate configured safety rules; issue prescriptions; record dispensing/administration separately.	P0/P1
FR-014–FR-018	Service order and execution	Create orders; allocate rooms, staff and resources; record execution status and result.	P0
FR-019	Diagnostic request	Support an approved manual request/result attachment or later adapter.	P1
FR-020–FR-021	Encounter conclusion	Record conclusion and follow-up; support configured leave/discharge review where applicable.	P0/P1
FR-022–FR-023	Catalogue, invoice and payment	Maintain effective service/price data; create invoices and retain payment/refund history.	P0
FR-024–FR-025	Notification and patient portal	Send configured operational notifications; support verified portal self-service only when enabled.	P1 / Phase 2
FR-026–FR-027	Dashboard, reports and export	Present scoped operational indicators and log controlled report exports.	P0
FR-028–FR-031	Administration, attachment and search	Maintain reference catalogues, controlled attachments, audit evidence and scoped search.	P0
FR-032	Draft and recovery	Preserve drafts and unsaved-change state; report success only after server confirmation.	P1
FR-033–FR-034	Facility and external integration	Apply approved facility scope and provide logged connectors only for approved contracts.	P2

3.6 Non-functional Requirements

The table defines the quality targets used for design and later verification.

Table 17: Non-functional requirements catalogue

ID	Attribute	Requirement / acceptance basis	Status
NFR-01–NFR-03	Performance, availability and recovery	Target p95 ordinary screen $\leq$ 2 seconds, search $\leq$ 3 seconds; define availability, RPO and RTO before deployment.	Target
NFR-04–NFR-07	Security and auditability	Use protected transport/storage, server-side RBAC, least privilege, time-synchronised sensitive-event evidence and retention policy.	Specified
NFR-08–NFR-09	Resilience and scale	Preserve confirmed data through dependency failure; use bounded queries, pagination and modular APIs.	Specified
NFR-10–NFR-11	Compatibility and accessibility	Support current Chrome/Edge desktop/tablet use with keyboard operation, labels, contrast and clear errors.	Target
NFR-12–NFR-13	Maintainability and observability	Use controlled migrations, documentation and structured operational metrics/logs.	Specified
NFR-14–NFR-15	Locale and integrity	Configure Vietnamese/local formats; use transactions, constraints and idempotency for critical operations.	Specified
NFR-16	Testability	Maintain unit, integration and end-to-end tests with non-production data.	Specified

3.7 Role-based Access Control Matrix

Table 18 records the intended minimum business permissions. It is a requirements baseline, not an implementation claim. **R**, **C**, **U**, **S**, **A** and **F** respectively mean read, create, update, sign/finalise, approve/override and finance transaction. A dash means no routine access. All non-dash permissions remain subject to facility, assignment and lifecycle state; Section 4 must enforce them at the server boundary.

3.8 Requirements Traceability and Use Case Validation

Table 19 links the business rationale to requirements, use cases, entities and planned verification.

Table 19: Requirements traceability and planned verification

Business rule/need	Requirement	Use cases	Domain/design focus	Planned verification
BR-001 identity continuity	FR-003, FR-004, FR-031	UC-010–UC-018	Patient, MedicalRecord, AuditLog; unique identifier and merge history.	Duplicate-search, unauthorised-merge and merge-history test cases.

Business rule/need	Requirement	Use cases	Domain/design focus	Planned verification
BR-002 conflict-free allocation	FR-005, FR-015–FR-017; NFR-15	UC-021–UC-026, UC-039	Appointment, ResourceBooking, schedule/room/staff constraints.	Concurrent booking/reschedule tests; zero accepted conflict target.
BR-003 controlled arrival	FR-006	UC-027–UC-030	Appointment–Encounter relationship and queue state.	Duplicate-active-encounter and role-scoped queue tests.
BR-005 signed-record integrity	FR-009, FR-032; NFR-07, NFR-15	UC-036–UC-038, UC-041–UC-042	ClinicalNote draft/signed/amendment lifecycle.	Missing-field, amendment and concurrent-edit tests.
BR-006 bounded warning	FR-011, FR-012	UC-043–UC-048	Prescription, PrescriptionItem, Medication and override evidence.	Rule-trigger, override-reason and no-autonomous-decision tests.
BR-007 accountable execution	FR-014–FR-018	UC-039–UC-040	ProcedureOrder, ResourceBooking and execution result.	Assignment, unavailable-resource, delay/cancel and result tests.
BR-008 separate medicine event	FR-012, FR-013	UC-048–UC-051	PrescriptionItem versus MedicationAdministration.	Verify issue alone creates no administration event.
BR-009 financial history	FR-022, FR-023; NFR-15	UC-060–UC-069	Invoice, Payment and refund/adjustment history.	Partial-payment, duplicate-submission and refund-reason tests.
BR-004 / BR-010 governance	FR-001, FR-002, FR-027, FR-030; NFR-06, NFR-07	UC-001, UC-070–UC-079	UserAccount, Role, AuditLog and controlled export.	Negative API authorisation and audit-completeness tests.
Operational visibility	FR-026, FR-027	UC-075–UC-076	Scoped measures, report filters and export audit.	Role-scope/report-reconciliation tests.

Open decisions. The clinic will confirm clinical-warning sources, consent and retention policy, refund approval thresholds, branch scope and portal publication rules before deployment.

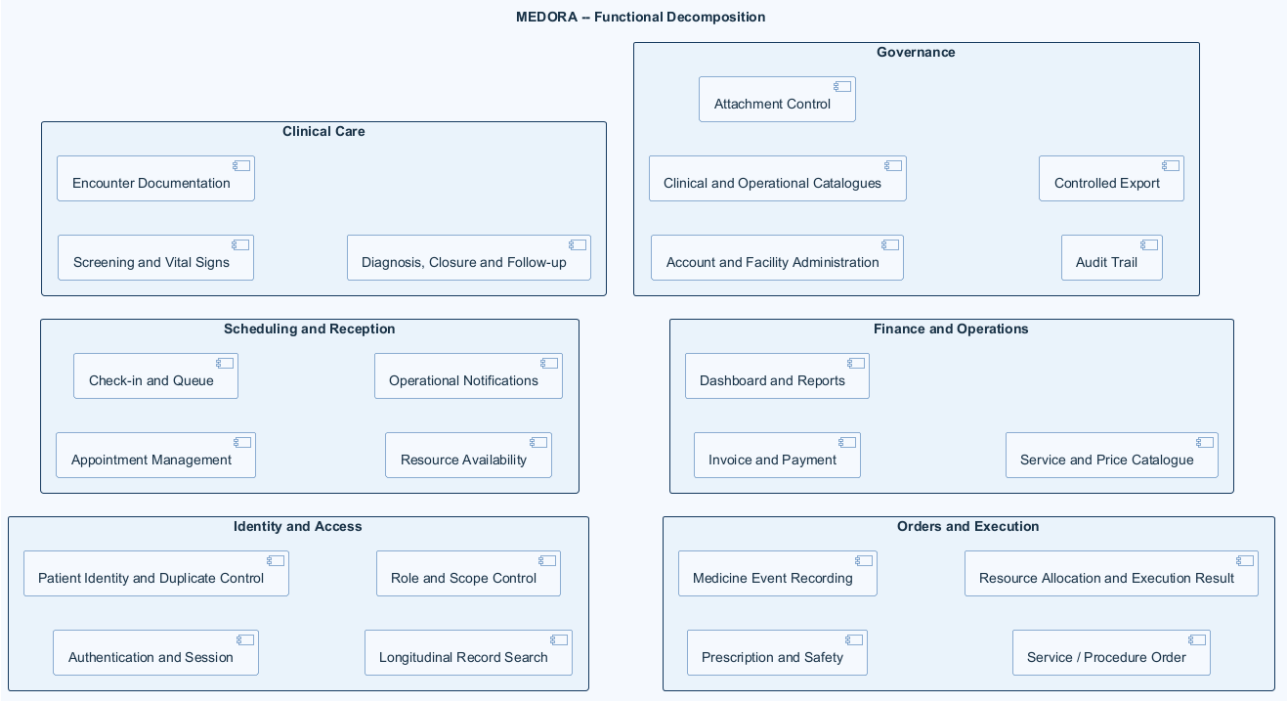


Figure 4: Functional decomposition of MEDORA

Clinical and front-desk roles

Capability / data domain	Rec.	Nurse	Doctor	Tech.	Pharm.
Patient administrative profile	R/C/U	R	R	R†	R†
Appointment, check-in and queue	R/C/U	R	R	R†	–
Screening and vital signs	R	C/U	R	R†	–
Clinical note and diagnosis	R†	R†	R/C/U/S	R†	R†
Prescription and medicine event	R†	R†	R/C/U/S/A	–	R/C/U
Service order and execution result	R†	R/C/U†	R/C/U/S	R/C/U	–
Invoice, payment and refund	R†	–	R†	–	–
Dashboard and scoped reports	–	R†	R†	R†	–
User, role and catalogue configuration	–	–	R†	–	R†
Audit log and controlled export	–	–	R†	–	–

Finance, governance and patient roles

Capability / data domain	Cash.	Manager	Admin	Auditor	Patient*
Patient administrative profile	R†	R†	–	R†	R/U
Appointment, check-in and queue	R†	R	–	R†	R/C/U
Screening and vital signs	–	R†	–	R†	–
Clinical note and diagnosis	–	R†	–	R†	R‡
Prescription and medicine event	–	R†	–	R†	R‡
Service order and execution result	–	R†	–	R†	R‡
Invoice, payment and refund	R/C/U/F	R/A†	–	R†	–
Dashboard and scoped reports	R†	R	–	R†	–
User, role and catalogue configuration	R†	R/U†	R/C/U/A	R†	–
Audit log and controlled export	R†	R†	R†	R	–

Table 18: Target role-based access-control matrix

*Phase 2 only. †Only within assigned, facility or approved scope. ‡Only information expressly released through the portal. Administrative configuration does not confer clinical-edit authority; Auditor access is read-only and itself may be logged.

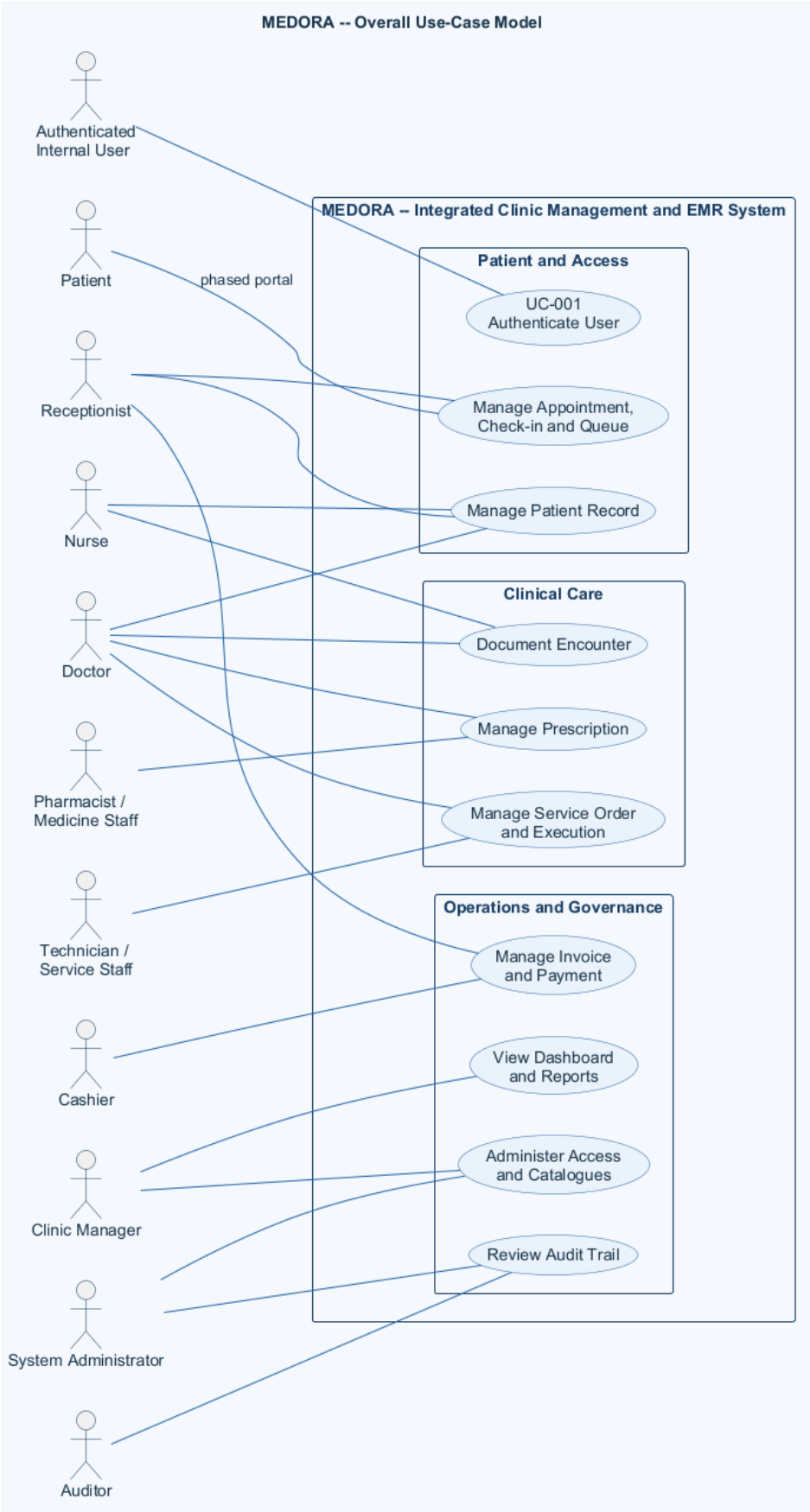


Figure 5: Overall use-case model of MEDORA

4 System Design and Structural Modeling

4.1 System Architecture

4.1.1 Logical Architecture

4.1.2 Deployment Architecture

4.2 Layered Design and Design Patterns

4.2.1 Presentation, Business Logic, and Data Access Layers

4.2.2 API and Web Service Design

4.3 Database Design and Data Integrity

4.3.1 Entity–Relationship Diagram

4.3.2 Relational Schema and Normalization

4.3.3 Persistent Classes and Database Mapping

4.3.4 Keys, Constraints, Data Types, and Multiplicities

4.4 Structural UML Diagrams

4.4.1 Overall Class Diagram

4.4.2 Class Diagrams by Use Case Group

4.4.3 Package Diagram

4.4.4 Component Diagram

4.4.5 Deployment Diagram

4.4.6 Object Diagram (if required)

4.5 Use Case-oriented Design

4.5.1 Participating Classes and Their Responsibilities

4.5.2 Collaboration Between System Components

5 Behavioral Modeling and System Implementation

5.1 Behavioral UML Diagrams

The behavioral models in this section refine the business rules and use cases in Sections 2 and 3. They describe proposed observable system behavior, not calls observed in an implemented application. The six system-sequence diagrams use MEDORA as a black-box boundary so that no unapproved controller, repository, API, or persistence technology is implied. The activity

and state models focus on exception handling, lifecycle integrity, and separation of clinical and financial records.

5.1.1 Sequence Diagrams for Key Use Cases

The sequence views cover appointment booking (UC-021), patient check-in (UC-027), signing and amending a clinical note (UC-037/UC-038), prescription issue (UC-048), service scheduling and execution (UC-039/UC-040), and payment/refund (UC-063/UC-065). Rejected operations leave no partial business effect; successful transitions record the relevant accountable event. The request key in the payment view is a proposed idempotency control derived from NFR-15, not an existing API contract.

5.1.2 Activity Diagrams for Complex Workflows

Figure 21 shows the service lifecycle from clinician order through resource validation and assigned execution. A conflict leaves the order pending without reserving resources; incomplete results do not mark execution complete. Figure 22 makes the boundary between signed clinical completion and invoice settlement explicit: an outstanding balance does not alter the clinical record. These diagrams complement the outpatient and appointment exception workflows in Figures 1 and 2.

5.1.3 State Machine Diagrams for Lifecycle-driven Entities

The state machines formalize the status proposals in WorkStation Sections 5 and 9 and the domain distinctions in Section 2. They are design constraints for later implementation and policy review. In particular, rescheduling preserves the prior appointment history, a clinical-note correction creates a linked version, and refunds remain separate financial transactions.

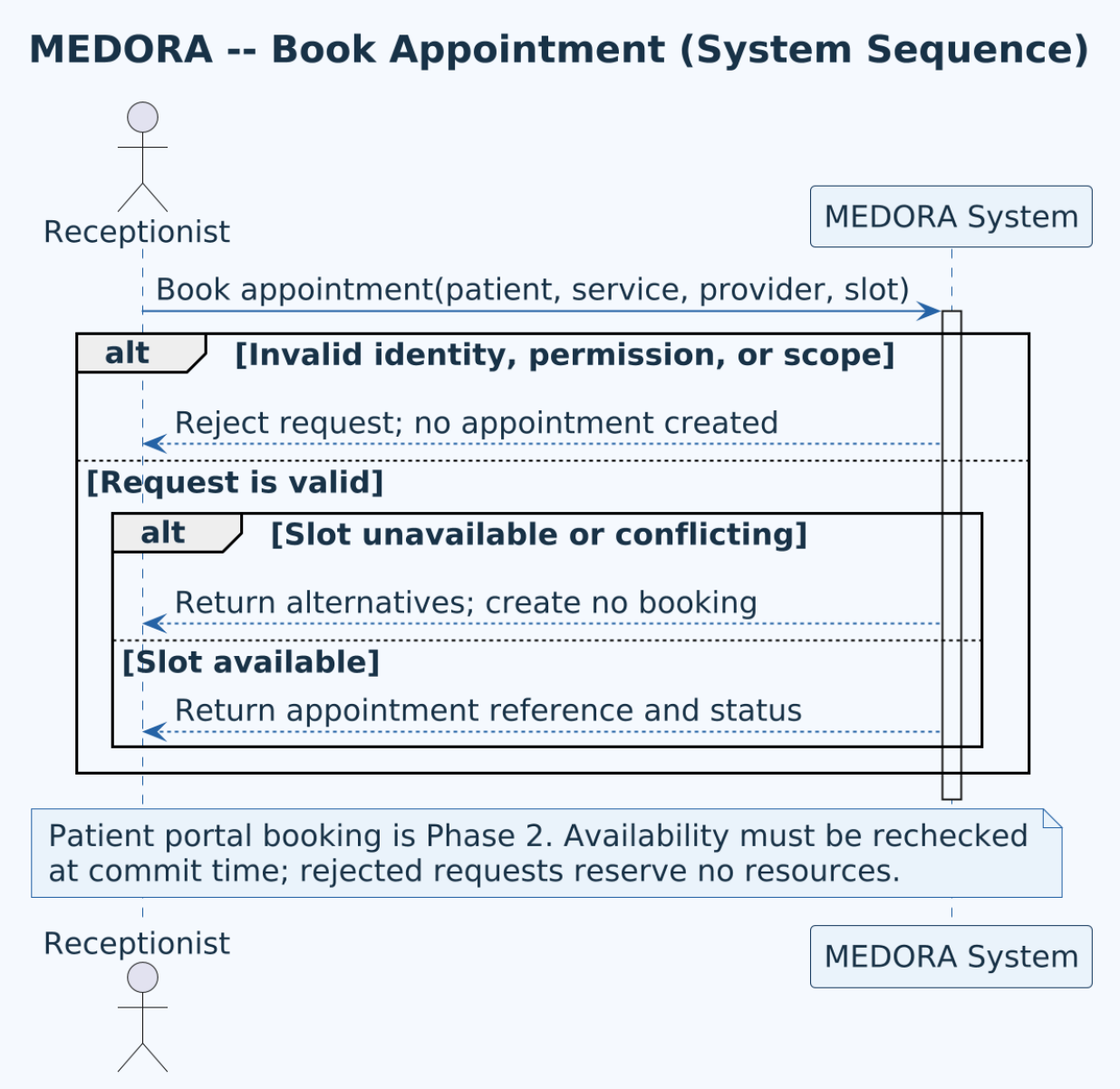


Figure 15: System sequence for appointment booking and availability validation

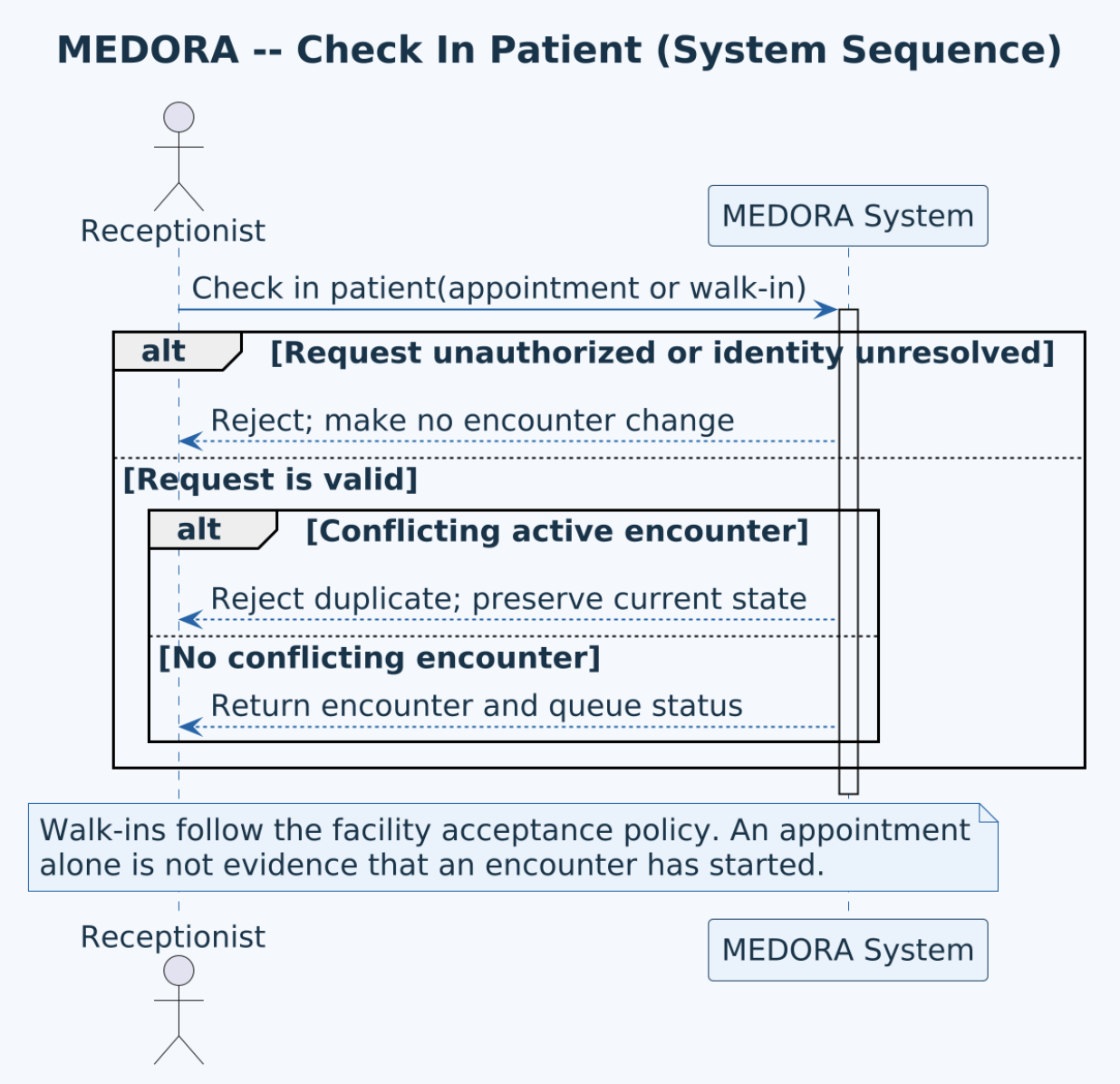


Figure 16: System sequence for check-in, encounter creation, and queue assignment

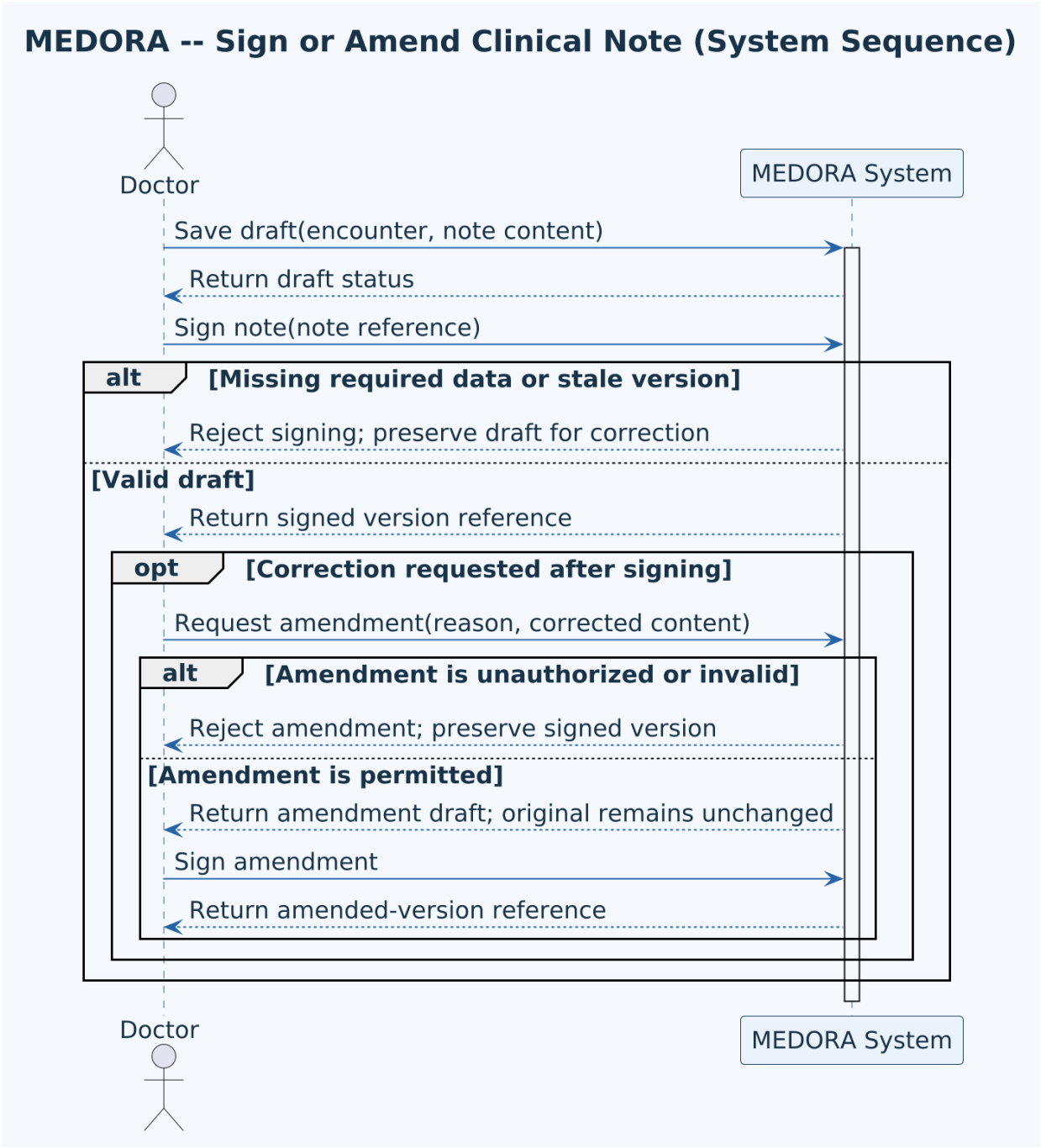


Figure 17: System sequence for signing a clinical note and appending a correction

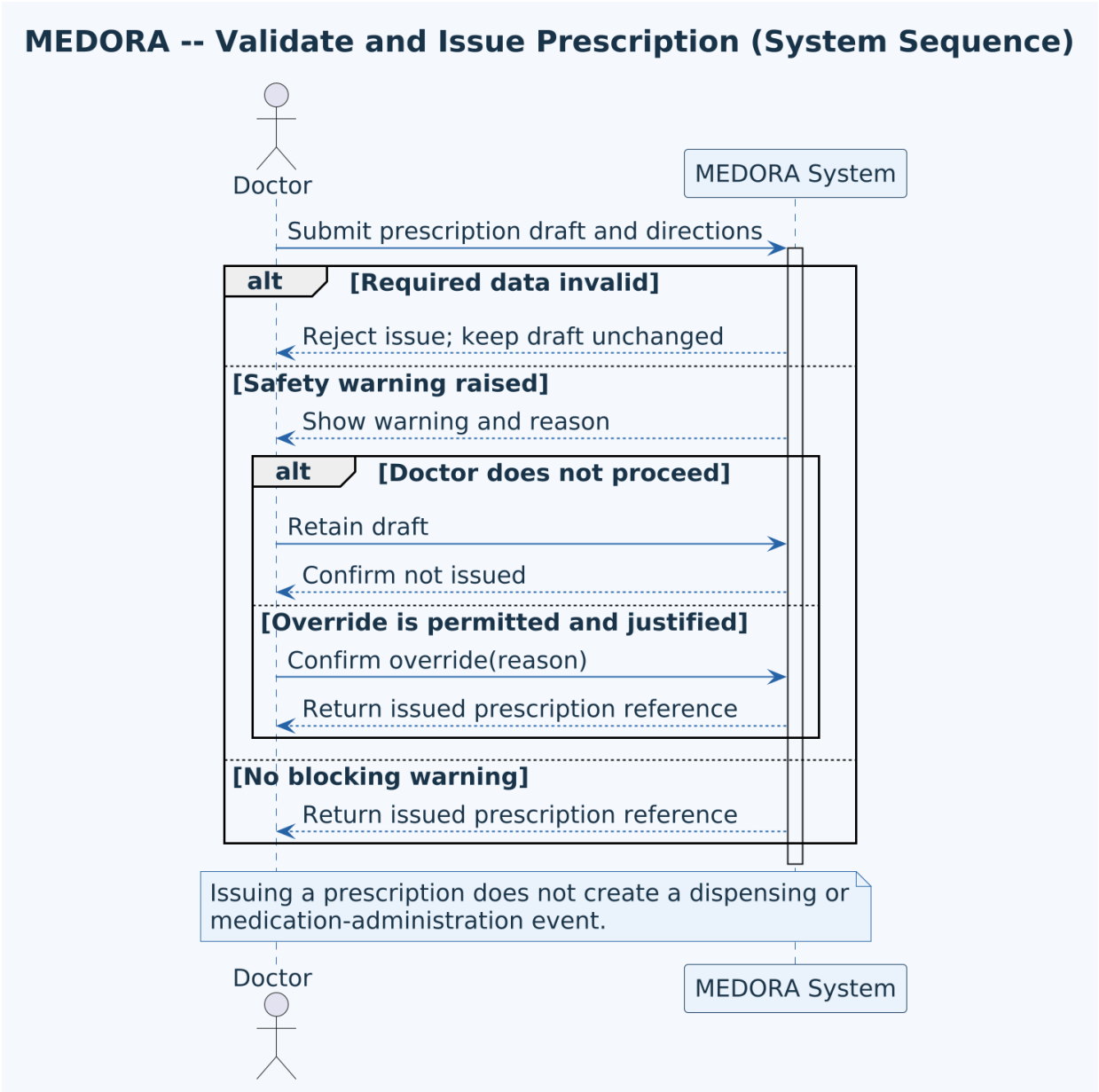


Figure 18: System sequence for configured prescription safety checks and issue

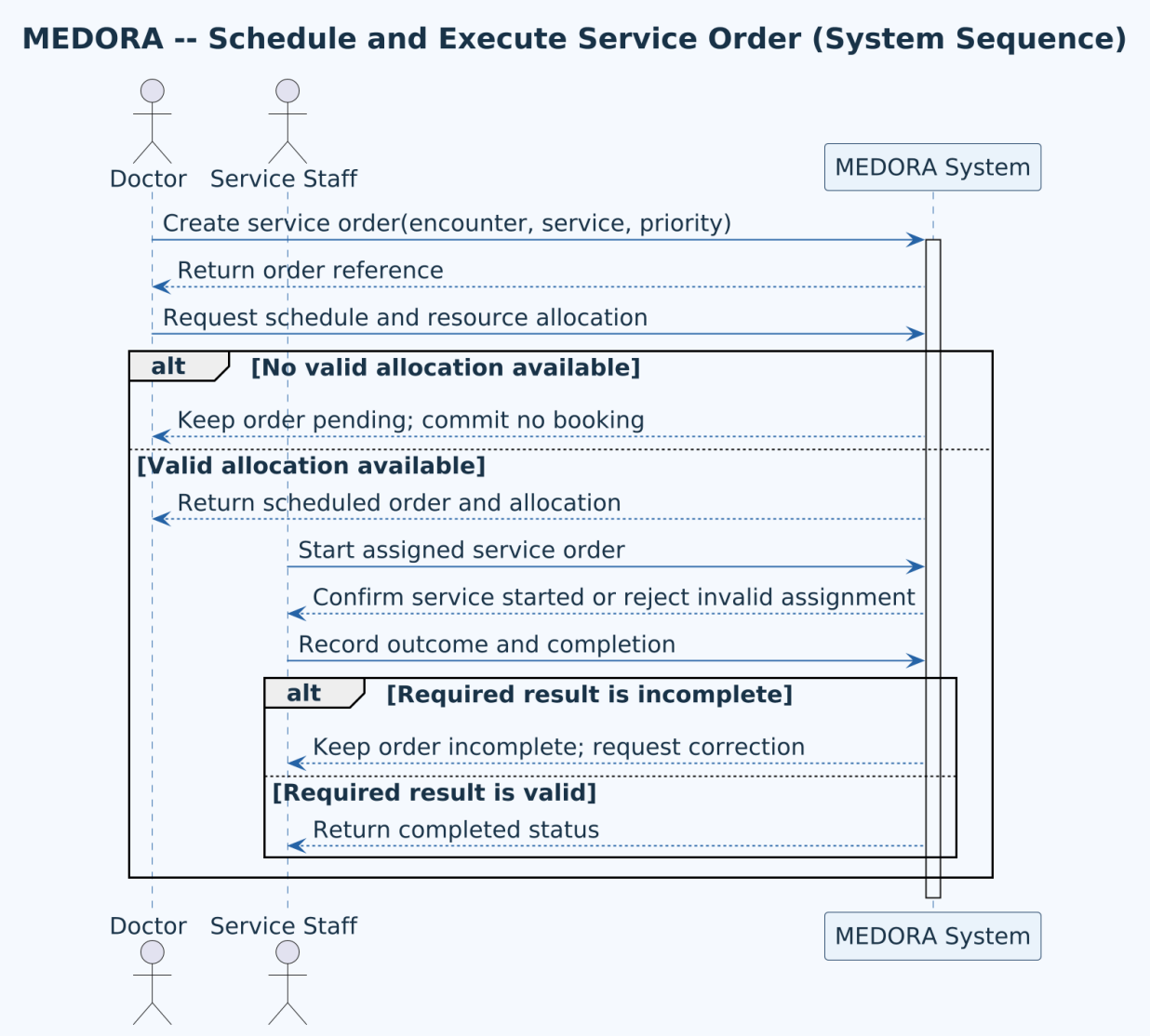


Figure 19: System sequence for resource allocation and service-order execution

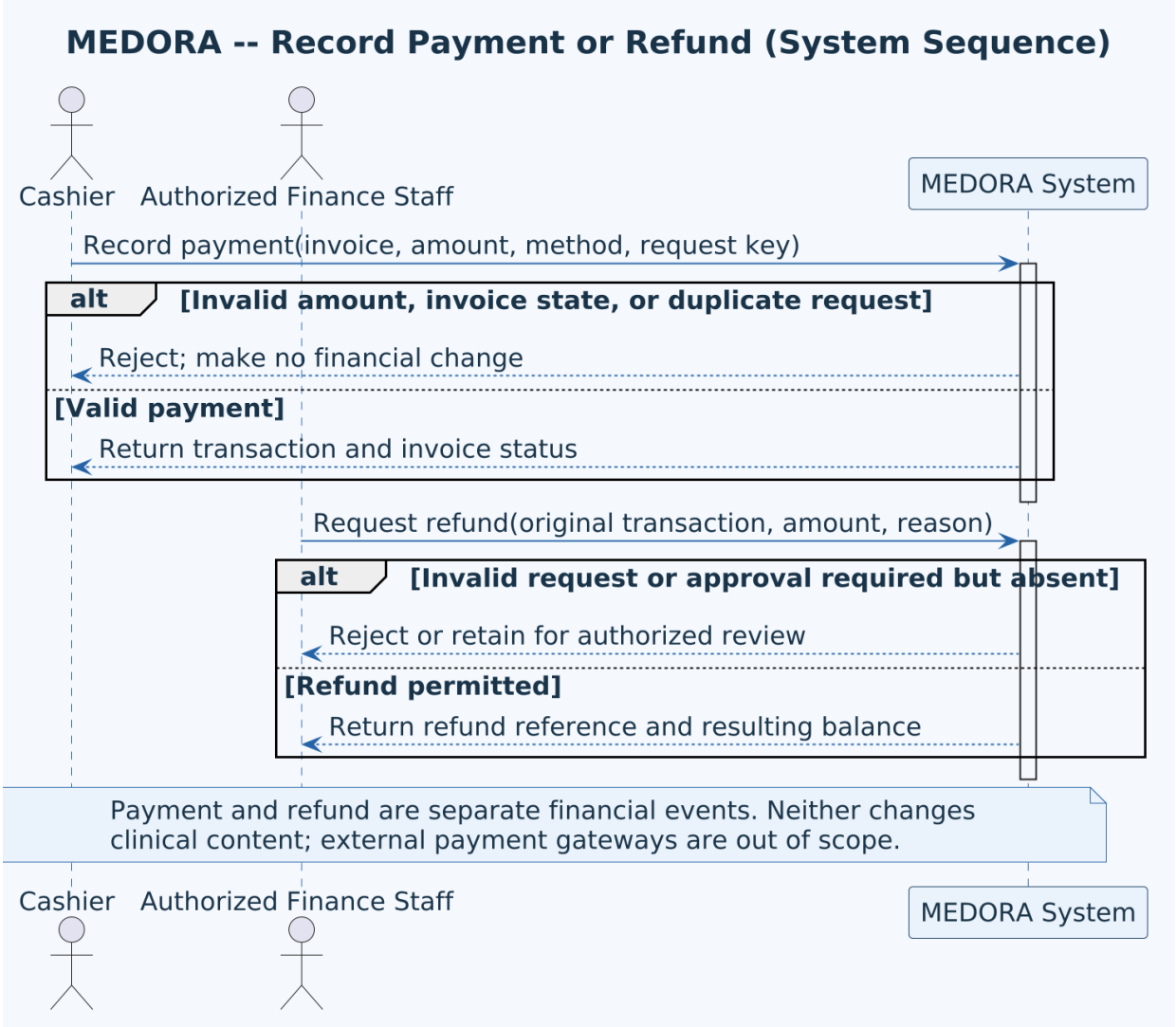


Figure 20: System sequence for payment and separately authorized refund events

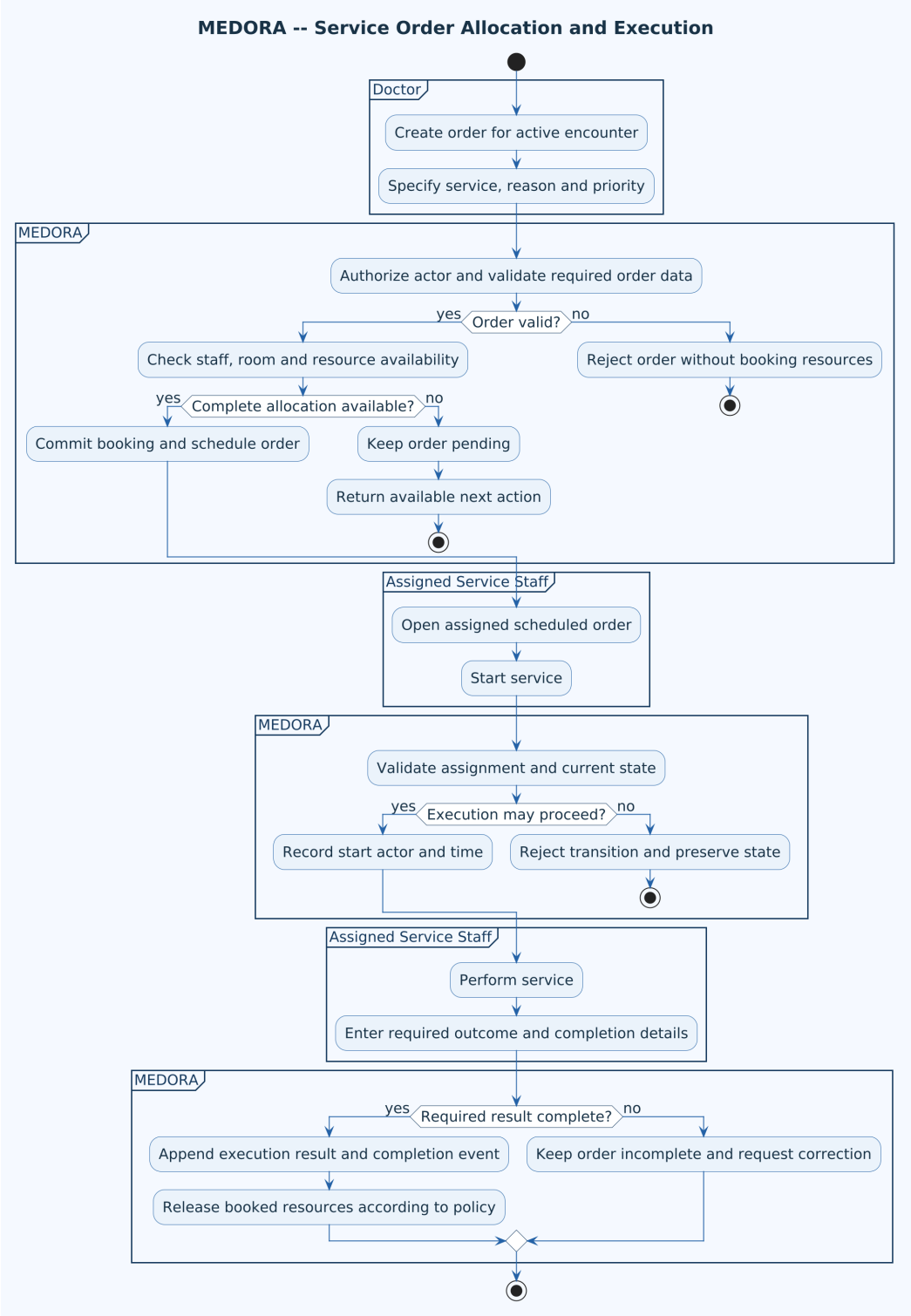


Figure 21: Activity model for service-order allocation and execution

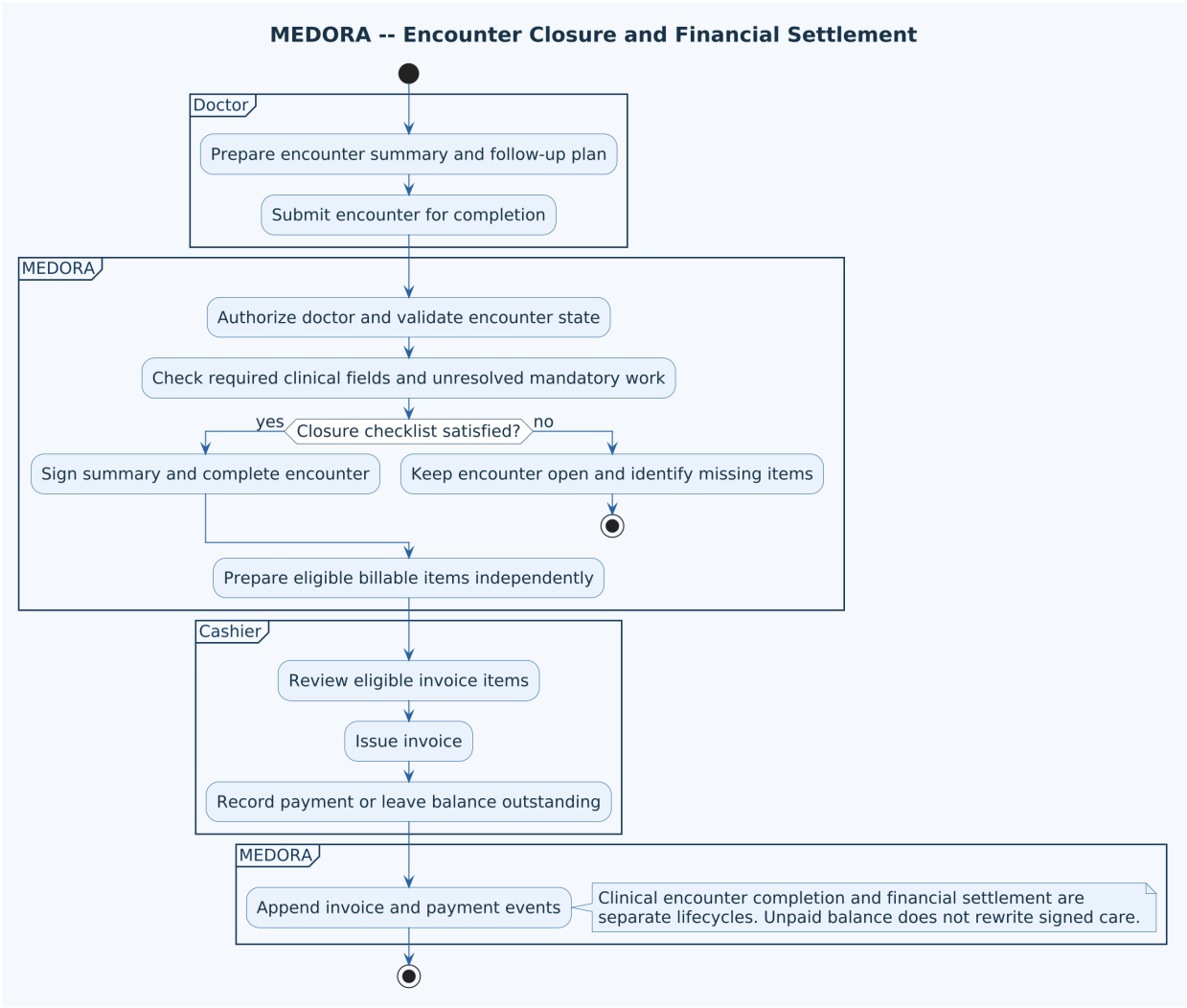


Figure 22: Activity model for encounter closure and independent financial settlement

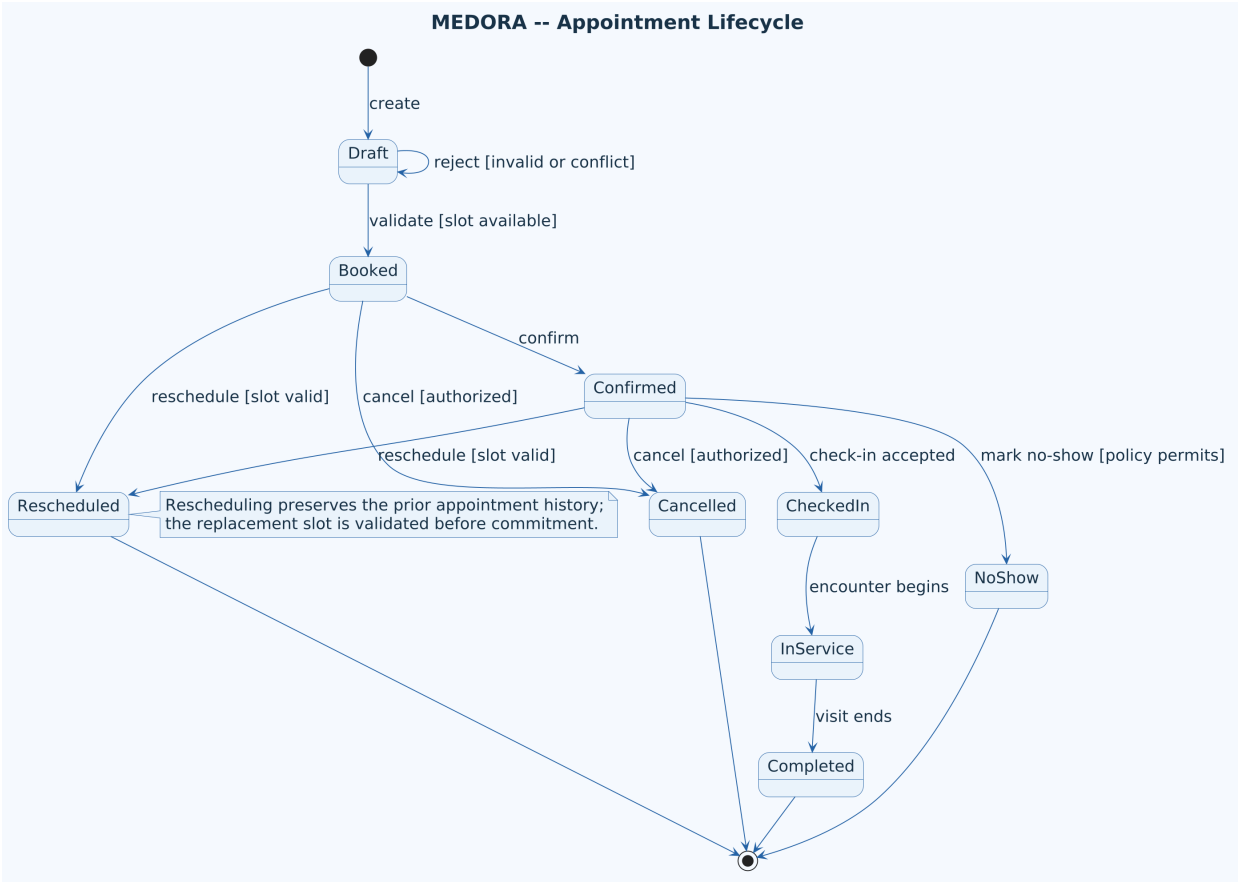


Figure 23: Proposed appointment lifecycle and terminal outcomes

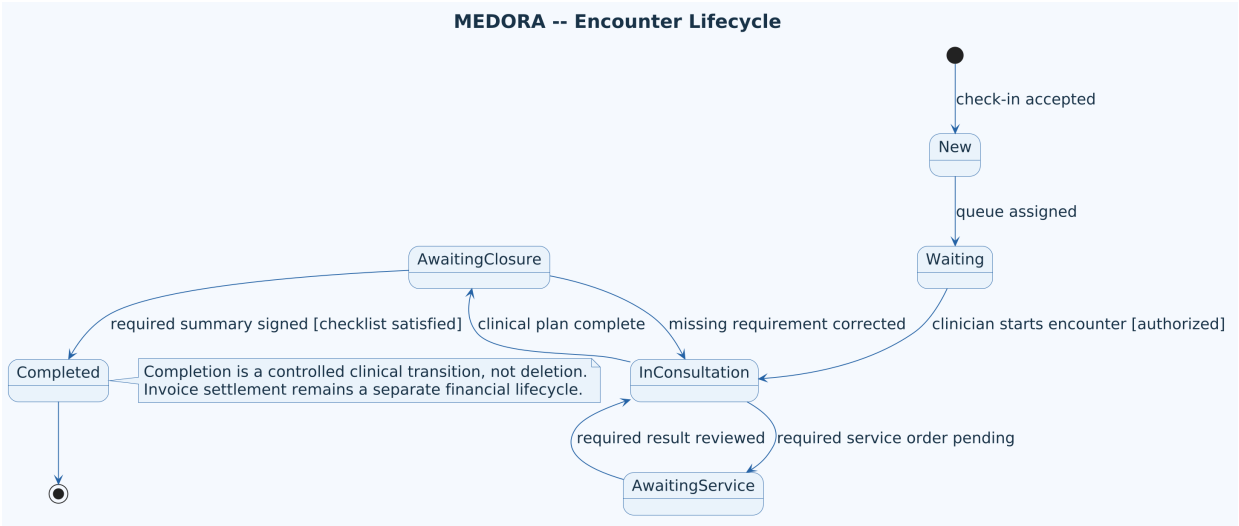


Figure 24: Proposed encounter lifecycle from check-in to clinical completion

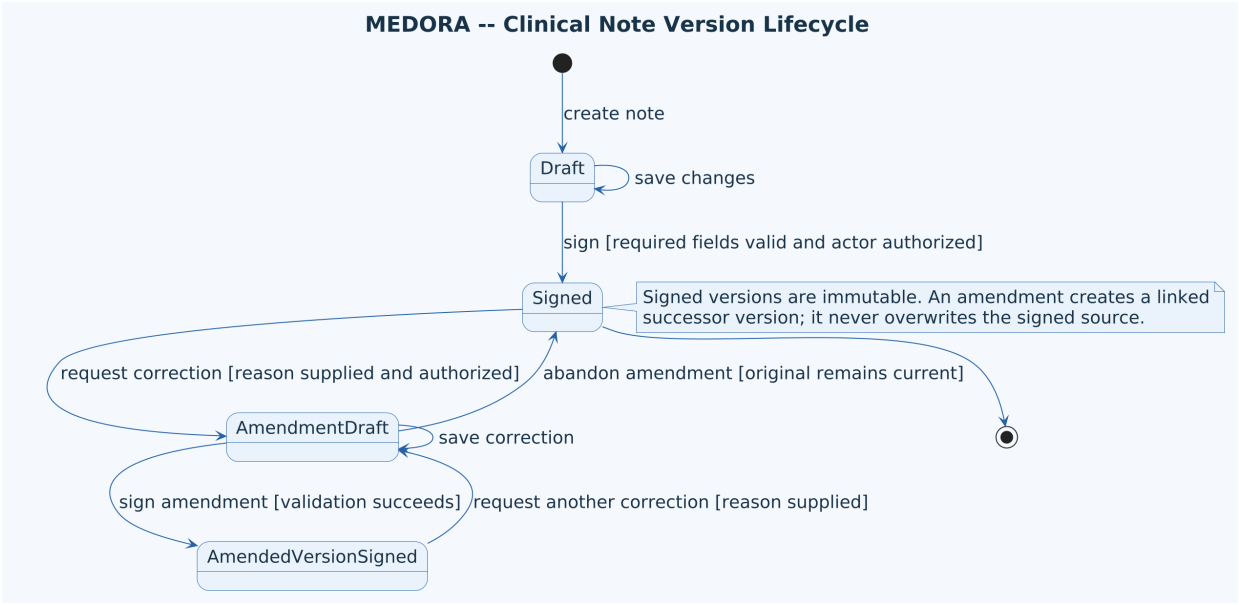


Figure 25: Clinical-note version lifecycle with immutable signed versions

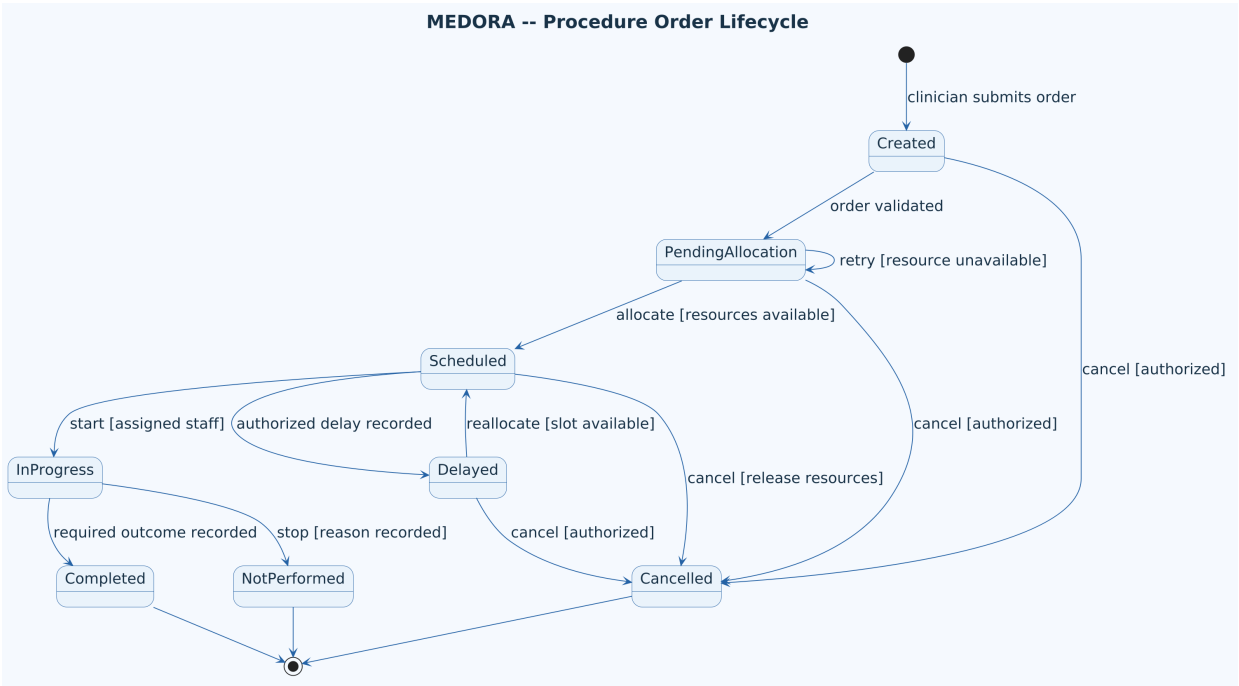


Figure 26: Proposed procedure-order lifecycle with allocation and execution outcomes

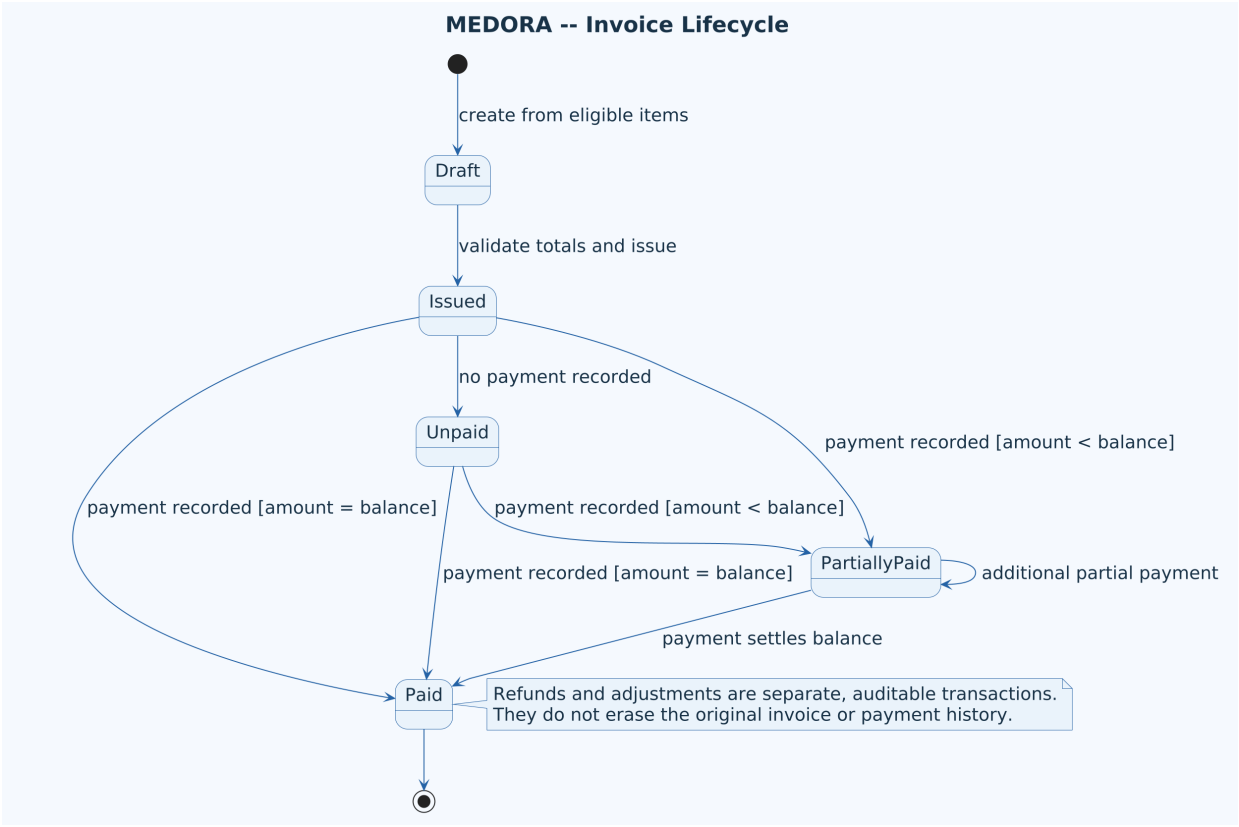


Figure 27: Proposed invoice lifecycle for unpaid, partial-payment, and paid states

5.2 Source Code Organization and Module Structure

The repository does not contain an application implementation from which a source-module architecture can be reported. The observable project structure is a report, its editable analysis/design assets, and a placeholder source directory. Table 20 records this boundary; proposed modules must be mapped to real files only after the implementation technology and architecture are approved.

Evidence item	Repository observation	Status / implication
Application source	`src/README.md` is a placeholder asking for source code.	No application modules or implemented behavior are evidenced.
Build/dependency manifest	No application package manifest or runtime build configuration is present.	Framework, language, and module boundaries are not established.
Persistence and API	No database schema, migration, API contract, or service implementation is present.	Data and endpoint behavior remain specified requirements, not implementation facts.
Behavioral assets	PlantUML sources and rendered figures are stored under `doc/asset/05-behavioral-modeling-implementation/`.	These are proposed behavioral models; they are not executable application code.
Deployment	No container, server, environment, or deployment configuration is present.	Production topology, hosting, and operational controls are not yet determined.

Table 20: Implementation and deployment evidence available in the repository

The report's conceptual layers and functional groups provide inputs for a future source organization, but they do not prove that presentation, application, domain, or persistence modules have been created. The next design step is to select a proportionate architecture, record its decision and dependencies, then map each component to actual source files and tests.

5.3 Implementation of Core Functionalities

No core capability can be marked implemented from the available repository evidence. Table 21 separates specification coverage from implementation status so that the planned scope is not mistaken for delivered software.

Capability area	Specification reference	Implementation evidence
Identity, roles, and patient records	FR-001–FR-004, FR-028–FR-031	Not present in source code; not verified.
Appointments, check-in, and queues	FR-005–FR-006, FR-017	Not present in source code; not verified.
Clinical documentation and encounter closure	FR-007–FR-010, FR-020–FR-021, FR-032	Not present in source code; not verified.
Prescription and medicine events	FR-011–FR-013	Not present in source code; not verified.
Service orders and resource execution	FR-014–FR-019	Not present in source code; not verified.
Billing, reporting, and audit	FR-022–FR-027, FR-030	Not present in source code; not verified.

Table 21: Specification coverage versus implementation evidence

5.4 Development and Deployment Environment

The environment available for this report is not the runtime environment of MEDORA. The report has been built locally with XeLaTeX/MiKTeX; behavioral figures were rendered from PlantUML through Kroki. These tools validate and publish documentation artifacts only. No application runtime, database, deployment target, environment variables, or operational service has been established, so this section makes no claim about application hosting or production readiness.

5.5 Implemented User Interfaces and API Evidence

No application UI, API, or operational workflow screenshot is available because no application source or running system is present. The workflow figures in this chapter are UML design evidence, not screenshots of implemented screens. No mock screen is substituted for implementation evidence.

Evidence required before implementation claims

Capture redacted screenshots from a running non-production build and identify the build/version, actor role, workflow state, and test case for each image. API evidence should include the approved endpoint and schema, authorization context, sanitized request/response, and observed result. Do not include real patient data or secrets.

6 System Testing and Evaluation

6.1 Testing Strategy and Test Environment

Testing is planned around the business risks and acceptance criteria in WorkStation and Sections 2–3. The strategy combines workflow, authorization, integrity, concurrency, and quality-attribute verification; it does not treat a rendered diagram or a successful report build as evidence that the application works.

The repository contains no application under test, database, API, automated test suite, test dataset, or deployment environment. Consequently, the test environment is not yet established and execution is blocked. When an application exists, testing should use an isolated non-production environment, synthetic patient data, controlled user roles, reproducible configuration, and a recorded build identifier. Performance and recovery checks additionally require an agreed workload and a provisioned environment representative of the intended deployment.

Test level	Planned purpose	Current evidence
Unit / domain	Validate lifecycle guards, calculations, authorization decisions, and invariants.	Not available; no implementation or unit tests.
Integration	Verify transactions across encounter, order, resource, invoice, payment, and audit boundaries.	Not available; no database or service integrations.
End-to-end workflow	Exercise the reception-to-encounter, clinical, service, and financial paths by role.	Not available; no running application or test environment.
Security / negative access	Send unauthorized requests directly to protected server operations and verify safe rejection.	Not available; no API or identity implementation.
Quality attributes	Measure response time, accessibility, locale behavior, availability, and restore objectives.	Not available; targets remain proposed and unmeasured.

Table 22: Planned verification levels and present evidence boundary

6.2 Test Scenarios and Test Case Design

The following test cases translate high-risk requirements and use cases into executable checks. Expected outcomes are specifications for future tests; the status column records that none has been executed.

Table 23: Planned test cases and current execution status

ID	Scenario and expected result	Trace	Status
TC-001	Call a protected patient/clinical operation without the required role or scope; reject the request without returning protected data or applying a state change.	FR-001–FR-002; NFR-06; UC-001	Not Run
TC-002	Register a patient matching an existing identity; flag a possible duplicate and prevent an unauthorized merge.	FR-003–FR-004; BR-001; UC-010–UC-018	Not Run
TC-003	Submit concurrent bookings for the same constrained provider, room, or resource; accept no conflicting allocation and preserve rejected requests without a booking.	FR-005, FR-015, FR-017; NFR-15; UC-021–UC-026	Not Run
TC-004	Check in an accepted walk-in and attempt a second active encounter for the same patient; create one valid encounter/queue and reject the duplicate.	FR-006; BR-003; UC-027–UC-030	Not Run
TC-005	Sign a note with missing required content, then amend a signed note; block incomplete signing and retain the original signed version with a linked, reasoned amendment.	FR-009, FR-020, FR-032; NFR-07; UC-037–UC-042	Not Run
TC-006	Issue a prescription with a configured safety warning; require the permitted response/override evidence and verify issue alone creates no dispensing or administration event.	FR-011–FR-013; BR-006, BR-008; UC-043–UC-052	Not Run
TC-007	Allocate a service order with unavailable or conflicting resources, then complete a valid execution; do not commit a conflict and do not mark complete without required results.	FR-014–FR-018; NFR-15; UC-039–UC-040	Not Run
TC-008	Record partial and repeated payment submissions; maintain the correct balance and prevent a duplicate request from applying a second financial effect.	FR-022–FR-023; NFR-15; UC-060–UC-069	Not Run
TC-009	Request a refund with invalid authority, amount, or reason; reject or hold it for approved review and retain the original transaction history.	FR-023, FR-030; BR-009; UC-065–UC-066	Not Run
TC-010	Export a scoped report and review its audit evidence; enforce the actor's scope and record the controlled export event.	FR-026–FR-027, FR-030; NFR-06–NFR-07; UC-076–UC-079	Not Run
TC-011	Measure ordinary-screen and search response times under an agreed representative workload; compare p95 values with the provisional targets.	NFR-01; WorkStation Section 10	Not Run
TC-012	Restore a protected backup and measure data loss and recovery time against approved RPO/RTO; a backup log alone is insufficient evidence.	NFR-02–NFR-03; WorkStation Section 10	Not Run
TC-013	Verify keyboard operation, labels, contrast, Vietnamese text, configured date/time, and currency formats on supported browsers.	NFR-10–NFR-11, NFR-14	Not Run

6.3 Functional Testing and Use Case Coverage

The planned cases cover representative paths across identity/access, scheduling/reception, clinical documentation, prescribing, service execution, finance, and governance. They are a first execution set, not proof of exhaustive FR-001–FR-034 coverage. Before implementation acceptance, each requirement and detailed use case must be mapped to one or more test cases, including negative authorization and exception cases.

Requirement group	Representative tests	Coverage status
FR-001–FR-004, FR-028–FR-031	TC-001–TC-002, TC-010	Specified; implementation and execution not evidenced.
FR-005–FR-006, FR-017	TC-003–TC-004	Specified; implementation and execution not evidenced.
FR-007–FR-010, FR-020–FR-021, FR-032	TC-004–TC-005	Specified; implementation and execution not evidenced.
FR-011–FR-019	TC-006–TC-007	Specified; implementation and execution not evidenced.
FR-022–FR-027, FR-030	TC-008–TC-010	Specified; implementation and execution not evidenced.
NFR-01–NFR-16	TC-001, TC-003, TC-005–TC-013	Verification methods proposed; no measurements or results available.

Table 24: Representative functional and quality-requirement test coverage

6.4 Exception Handling, Authorization, and Data Integrity Testing

The highest-risk checks are rejection paths that must leave no invalid partial state: unauthorized access (TC-001), duplicate identity/encounter handling (TC-002 and TC-004), concurrent resource allocation (TC-003 and TC-007), incomplete signing and append-only amendments (TC-005), bounded prescription overrides (TC-006), and idempotent payment/refund handling (TC-008–TC-009). Expected behavior is specified, but the corresponding server-side controls and transactional boundaries cannot be verified until they exist in code.

6.5 Test Execution Results and Evidence

Execution status: Not Run

As of this report revision, there is no runnable MEDORA application, test harness, database, or configured test environment in the repository. All 13 cases in Table 23 are therefore **Not Run**. No case is reported as Pass or Fail, no coverage percentage is claimed, and no execution screenshot or log is available. Execution is blocked pending an implemented build and test environment.

Status	Count	Evidence
Pass	0	No executed results.
Fail	0	No executed results.
Blocked	0	The suite has not started; the prerequisite gap is recorded below.
Not Run	13	No system under test or execution environment.

Table 25: Test execution summary at report time

6.6 Evaluation Against Functional and Non-functional Requirements

Functional requirements are specified and traced to use cases, but none is verified as implemented. The quality targets below come from WorkStation and remain proposed acceptance criteria pending owner confirmation and measurement.

Evaluation measure	Proposed acceptance basis	Current evaluation
Registration-to-completion time	Reduce median by at least 25% against a documented baseline for the representative scenario.	Not measured; no baseline or executable workflow.
Conflicting allocations	Zero accepted conflicts in all defined concurrency cases.	Not measured; no scheduling implementation.
Encounter completeness	At least 95% of configured mandatory fields for tested completed visits.	Not measured; no configured forms or visits.
Sensitive-event traceability	100% of tested sensitive events have actor, time, action, and target evidence.	Not measured; no audit implementation.
Authorization	100% of negative authorization cases rejected at the server/API boundary.	Not measured; no API or access-control implementation.
Response time	p95 ordinary screens $\leq$ 2 seconds and search $\leq$ 3 seconds under an agreed workload.	Proposed only; no deployment or load test.
Availability and recovery	Provisional 99.5% monthly availability, RPO $\leq$ 15 minutes, and RTO $\leq$ 4 hours.	Proposed only; no operational environment or restore exercise.

Table 26: Evaluation criteria and evidence status

6.7 Limitations and Outstanding Issues

The principal evaluation limitation is absence of an implemented system. The proposed test cases cannot establish behavior, performance, security, accessibility, recovery, or user acceptance. The clinic must also confirm clinical templates and warning sources, retention and consent rules, merge/refund/override approvals, branch scope, and patient-portal release policy before those behaviors can be treated as definitive acceptance requirements. No legal or certification compliance is claimed.

7 Conclusion and Future Work

7.1 Summary of Achievements

The current deliverable establishes a clinic-focused business and requirements baseline, a traceable use-case model, and a set of behavioral models for appointment booking, check-in, clinical-note integrity, prescription safety, service execution, encounter closure, and payment history. It also defines a practical test plan with expected outcomes and explicit status reporting. These are analysis and design achievements; they do not demonstrate that MEDORA has been implemented, deployed, or tested.

7.2 Limitations of the Current System

There is no application implementation in the repository, so MEDORA should be described as a specified/proposed system rather than an operational product. No source modules, persistent schema, API, user interface, deployment topology, test execution evidence, or user feedback are available. The use-case, activity, sequence, and state diagrams express intended behavior; they remain subject to policy confirmation and later reconciliation with code. Performance, availability, recovery, and success-rate values are targets only. Clinical rule sources, data retention, consent, approval thresholds, multi-branch operation, and portal publication remain open decisions.

7.3 Future Improvements and Extensions

Future work should proceed in evidence-producing stages:

1. Confirm clinic policies for authorization, clinical safety, record correction, finance, retention, and lifecycle transitions.
2. Decide and document the architecture, persistence model, deployment assumptions, and mapping from approved components to source modules.
3. Implement a testable MVP for identity, appointment/check-in, encounter documentation, role/scope enforcement, and audit evidence.
4. Extend the MVP with clinician-approved prescription and service workflows, then basic finance with separate, immutable transaction history.
5. Establish automated and negative-authorization tests; measure quality targets using synthetic data and a recorded environment; capture privacy-redacted UI/API evidence and usability/recovery results.
6. Add the patient portal or external integrations only after scope, contracts, security controls, and operational ownership are approved.

This sequence keeps the report aligned with the project's clinic scope and makes each future claim auditable against source code, test results, or operational evidence.

A Detailed Use Case Specifications

B Supplementary UML Diagrams

C API Specifications and Data Schemas

D Detailed Test Cases and Execution Results

References

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